

RESPONSE TO REVIEW

**Response to queries following
site visit**

PCSAS Question 1: *We need to see full syllabi for the two core courses (including anticipated readings), as well as an outline of the way the class will unfold (i.e., estimated weekly topics), while recognizing that these will be modified later. If other core courses are added to address the issues raised in review (i.e., coverage of psychopathology research and intervention science), we need to see similar materials for these courses).*

Thank you for the opportunity to clarify and supplement our submission. In response to this request, we are submitting the full syllabi with anticipated readings and weekly topics for the Clinical Psychological Science concentration sequence. Each syllabus includes weekly topics and reading lists,¹ with the understanding that specific content may evolve as the courses are delivered and refined.

The Review Committee appropriately emphasized that the scholarship of psychopathology must be part of the required sequence and demonstrated for all students, not left to electives or lab-specific experiences. We agree and have revised our approach to the core sequence of CPSY 7300 and CPSY 7310 to address this need. Our department has a psychopathology course already available² in the graduate catalog (PSYC 7308: Psychopathology). We have (1) substantively revised PSYC 7308 to meet the needs of our clinical science concentration, and (2) we now require completion of PSYC 7308 as a prerequisite for enrollment in CPSY 7300.

Although PSYC 7308 appears as an elective on the doctoral degree plan, it functions as an essential part of the concentration sequence. All students in the Clinical Psychological Science concentration are required to take PSYC 7308, and the student handbook has been revised to make this explicit. Our student handbook (p. 12, “Electives”) has been revised to state:

All students in the Clinical Psychological Science concentration will take PSYC 7308 (Psychopathology) as one of their electives. This course serves as the foundational preparation for the concentration’s core sequence (CPSY 7300 and CPSY 7310) and is therefore a pre-requisite for enrollment in CPSY 7300. Although it appears as an elective on the degree plan, it functions as an essential part of the training sequence, ensuring that all students enter the concentration with a shared grounding in psychopathology scholarship.

In making revisions to PSYC 7308, we ensured comprehensive coverage of the psychopathology domains highlighted in the Committee’s letter, including epidemiology, genetics, neurobiology, cognitive and affective mechanisms, environmental/contextual risk, comorbidity, heterogeneity, developmental trajectories, and cultural factors. Weekly anchor readings are drawn from current primary research in leading journals (e.g., *JAMA Psychiatry*, *Nature Medicine*, *World Psychiatry*, *Lancet*), alongside dimensional nosology frameworks such as HiTOP and RDoC.

¹ Most assigned readings are sourced from peer-reviewed journal articles to ensure accessibility. To prevent any financial burden on students, the program has purchased institutional access to key resources, including the two-volume *APA Handbook of Psychotherapy* and the eleven-volume set *Comprehensive Clinical Psychology*.

² In our original submission, we interpreted PCSAS guidance to require syllabi only for newly developed concentration courses, rather than existing departmental offerings. Accordingly, PSYC 7308 was not included in our initial materials, though it has always been available within the doctoral curriculum.

Major assignments include an Integrative Psychopathology Analysis paper, oral presentation, and cumulative final exam, ensuring that students demonstrate doctoral-level mastery of psychopathology research. The revised syllabus is attached for your review (pp. 3-18).

In addition, we have revised our two concentration courses. CPSY 7300 now begins after 7308 and emphasizes philosophy of science, construct validity, psychometric theory, measurement-based care, mediators and moderators of change, structured interviews, and multicultural/ethical considerations. Major assignments include a Psychometrics Project that requires students to critically evaluate the psychometric and cultural adequacy of standardized measures, as well as a synthesis-based final exam and competency-based labs. CPSY 7310 has been restructured to provide a clear sequence through intervention science, beginning with outcome designs and measurement, and progressing through mediators and moderators of change, empirically supported treatments, process-based CBT, dissemination/implementation science, supervision, systems of care, and cultural/contextual adaptations. Students complete an NIH-style grant proposal and oral defense, along with cumulative final exams and labs focused on fidelity monitoring, rupture/repair practice, and outcome tracking.

Across the arc, assignments scaffold integration of scholarship and methodological critique: the Integrative Psychopathology Analysis in 7308, the Psychometrics Project in 7300, and the Grant Proposal in 7310, supplemented by cumulative exams. To reinforce competencies longitudinally, we also built in additional requirements: (a) a Year 1 summer interlude CBT knowledge exam before students carry cases, (b) a Year 2 summer interlude requiring completion of four online EST certifications (CPT, TF-CBT, CBT-I, Acceptance-Based Behavioral Therapy for GAD), and (c) ongoing fidelity monitoring and case presentations in the PSEC practicum.

With PSYC 7308 providing the foundational psychopathology training and CPSY 7300 and 7310 building sequentially upon it, all students acquire broad and deep preparation in psychopathology, assessment, and intervention, anchored in primary research literature and consistent with the PCSAS clinical science model. Finally, we have added a policy note (on p. 7 of student handbook) specifying that all core courses must be completed with a B or better, with any grade of B- or lower requiring the student to repeat the course. Full syllabi for CPSY 7300 (pp. 18-33) and CPSY 7310 (pp. 34-51) are provided below.

PSYC 7308: Psychopathology

Course Pre-requisites, Co-requisites, and/or Other Restrictions

Department consent required.

Course Description

Comprehensive overview of psychopathology as a clinical science discipline. The course emphasizes empirical and theoretical models of mental disorders, including their epidemiology, genetic and neurobiological underpinnings, cognitive and affective mechanisms, and environmental and cultural risk factors. Major disorder domains (e.g., mood, anxiety, trauma-related, psychotic, substance use, externalizing, and personality pathology) are examined primarily in adulthood, with developmental and lifespan considerations integrated throughout. Readings focus on primary research literature, with attention to comorbidity, heterogeneity, and dimensional nosology frameworks (e.g., HiTOP, RDoC) alongside traditional DSM/ICD classification systems. Historical and cultural perspectives on psychopathology are also addressed.

Student Learning Objectives/Outcomes

The learning activities assigned across this course are intended to develop critical thinking skills essential for evaluating psychological research and practice. Students will be able to:

- Critically evaluate primary-source empirical literature on major domains of psychopathology (mood, anxiety, trauma-related, psychotic, externalizing, and personality disorders).
- Integrate findings across multiple levels of analysis (epidemiology, genetics, psychophysiology, cognition/affect, and environmental risk factors) to explain the etiology and course of psychopathology.
- Compare and contrast categorical nosologies (DSM/ICD) with dimensional and transdiagnostic models (HiTOP, RDoC, network approaches) and analyze their implications for research and clinical science.
- Synthesize developmental and cultural/contextual perspectives into scholarly analyses of psychopathology.
- Demonstrate scholarly independence by producing written and oral integrative analyses that reflect doctoral-level facility with psychopathology research and theory.

Course Access and Navigation

This course can be accessed using your UT Dallas NetID account on the [eLearning](#) website. Please see the course access and navigation section of the [Getting Started with eLearning](#) webpage for more information. To become familiar with the eLearning tool, please see the [Student eLearning Tutorials](#) webpage. UT Dallas provides eLearning technical support 24 hours a day, 7 days a week. The [eLearning Support Center](#) includes a toll-free telephone number for immediate assistance (1-866-588-3192), email request service, and an online chat service.

Course Schedule & Readings

No textbook is required for purchase in this course. All readings will be provided on the course eLearning platform. Each week includes a set of required readings that collectively ensure the breadth and depth of coverage in psychopathology expected at the doctoral level. Within each week, certain readings are marked with * to indicate Anchor Readings. These are the pieces every student must be prepared to discuss in class, as they form the starting point the weekly seminar. The remaining readings are equally required and represent the broader scientific literature from which students are expected to draw when preparing written assignments, oral presentations, and the Integrative Psychopathology Analysis.

Fall 2026 Weekly Schedule

Week Beginning August 24: Orientation to Psychopathology as a Clinical Psychological Science

Definition and scope of psychopathology, methods and theoretical frameworks, integration and classification

- *Becker, E. S. & Rinck, M. (2022). Experimental psychopathology. In G. Asmundson (Ed.) *Comprehensive Clinical Psychology* (2nd ed., Vol. 1, pp. 124-125). Elsevier. <https://dx.doi.org/10.1016/B978-0-12-818697-8.00128-X>
- *Vaidyanathan, U., Vrieze, S. I., & Iacono, W. G. (2015). The Power of Theory, Research Design, and Transdisciplinary Integration in Moving Psychopathology Forward. *Psychological Inquiry*, 26(3), 209-230. <https://doi.org/10.1080/1047840X.2015.1015367>

Week Beginning August 31: Primer on Epidemiology

Incidence, prevalence, comorbidity, risk/protection; epidemiological study designs and interpretation.

- *Sommer, J. L., Grocott, B., & El-Gabalawy, R. (2022). An overview of epidemiology in clinical psychology. In G. Asmundson (Ed.) *Comprehensive Clinical Psychology* (2nd ed., Vol. 1, pp. 79-95). Elsevier. <https://dx.doi.org/10.1016/B978-0-12-818697-8.00100-X>
- *Caspi A, Houts RM, Ambler A, et al. Longitudinal Assessment of Mental Health Disorders and Comorbidities Across 4 Decades Among Participants in the Dunedin Birth Cohort Study. *JAMA Netw Open*. 2020;3(4):e203221. doi:10.1001/jamanetworkopen.2020.3221
- Köhler, C. A., Evangelou, E., Stubbs, B., et al. (2018). Mapping risk factors for depression across the lifespan: An umbrella review of evidence from meta-analyses and Mendelian randomization studies. *Journal of Psychiatric Research*, 103, 189-207. <https://doi.org/10.1016/j.jpsychires.2018.05.020>

Week Beginning September 7: Primer on Genetics and Environmental Risk

Twin/adoption studies, GWAS, gene × environment, epigenetics; environmental adversities (maltreatment, inequity, racism).

- *Mitchell, B. L., Thorp, J. G., Wu, Y., et al. (2021). Polygenic risk scores derived from varying definitions of depression and risk of depression. *JAMA Psychiatry*, 78(10), 1152-1160. <https://doi.org/10.1001/jamapsychiatry.2021.1988>
- *Musliner, K. L., Mortensen, P. B., McGrath, J. J., et al. (2019). Association of polygenic liabilities for major depression, bipolar disorder, and schizophrenia with risk for depression in the Danish

population. *JAMA Psychiatry*, 76(5), 516-525.
<https://doi.org/10.1001/jamapsychiatry.2018.4166>

- Brown, A. S. (2011). The environment and susceptibility to schizophrenia. *Progress in Neurobiology*, 93(1), 23-58. <https://doi.org/10.1016/j.pneurobio.2010.09.003>

Week Beginning September 14: Epidemiology & Construct Validity in Psychopathology

Measurement challenges, heterogeneity, construct validity; integrating biological, cognitive, and environmental levels.

- *Flake, J. K., & Fried, E. I. (2020). Measurement schmeasurement: Questionable measurement practices and how to avoid them. *Advances in methods and practices in psychological science*, 3(4), 456-465.
- *Feczko, E., Miranda-Dominguez, O., Marr, M., Graham, A. M., Nigg, J. T., & Fair, D. A. (2019). The heterogeneity problem: approaches to identify psychiatric subtypes. *Trends in Cognitive Sciences*, 23(7), 584-601.
- Fried, E. I., Flake, J. K., & Robinaugh, D. J. (2022). Revisiting the theoretical and methodological foundations of depression measurement. *Nature Reviews Psychology*, 1(6), 358-368.
- Anvari, F., Alsalti, T., Oehler, L., Hussey, I., Elson, M., & Arslan, R. C. (2024). A fragmented field: Construct and measure proliferation in psychology. *Preprint*. <https://osf.io/preprints/psyarxiv/b4muj>.
- MacCorquodale, K., & Meehl, P. E. (1948). On the distinction between hypothetical constructs and intervening variables. *Psychological Review*, 55, 95-107. (Especially try to capture the distinction as summarized from page 103 on).

Week Beginning September 21: Mood Disorders and Suicide Risk

Epidemiology and course of major depressive disorder and bipolar spectrum conditions; genetic architecture and polygenic risk; reward processing, cognitive control, and affective mechanisms; environmental and developmental risk factors (stress, maltreatment, inequity); suicide risk models and predictors; implications for classification (DSM/ICD vs dimensional models)

- *Uher, R., Pavlova, B., Najafi, S., et al. (2024). Antecedents of major depressive, bipolar, and psychotic disorders: A systematic review and meta-analysis of prospective studies. *Neuroscience & Biobehavioral Reviews*, 160, 105625. <https://doi.org/10.1016/j.neubiorev.2024.105625>
- *Als, T. D., Kurki, M. I., Grove, J., et al. (2023). Depression pathophysiology, risk prediction of recurrence and comorbid psychiatric disorders using genome-wide analyses. *Nature Medicine*, 29(7), 1832-1844. <https://doi.org/10.1038/s41591-023-02352-1>
- American Psychiatric Association. (2022). Depressive disorders. In *Diagnostic and Statistical Manual of Mental Disorders* (5th ed., text rev.; pp. 155-180). American Psychiatric Publishing.
- American Psychiatric Association. (2022). Bipolar and related disorders. In *Diagnostic and Statistical Manual of Mental Disorders* (5th ed., text rev.; pp. 123-154). American Psychiatric Publishing.
- Park, S. C., & Kim, Y. K. (2021). Challenges and strategies for current classifications of depressive disorders: Proposal for future diagnostic standards. In *Advances in Experimental Medicine and Biology* (Vol. 1305, pp. 103-116). Springer. https://doi.org/10.1007/978-981-33-6044-0_7
- McIntyre, R. S., Berk, M., Brietzke, E., et al. (2020). Bipolar disorders. *The Lancet*, 396(10265), 1841-1856. [https://doi.org/10.1016/S0140-6736\(20\)31544-0](https://doi.org/10.1016/S0140-6736(20)31544-0)

- Silva, R. C., Maffioletti, E., Gennarelli, M., Baune, B. T., & Minelli, A. (2021). Biological correlates of early life stressful events in major depressive disorder. *Psychoneuroendocrinology*, 125, 105103. <https://doi.org/10.1016/j.psyneuen.2020.105103>
- Agerbo, E., Trabjerg, B. B., Børglum, A. D., et al. (2021). Risk of early-onset depression associated with polygenic liability, parental psychiatric history, and socioeconomic status. *JAMA Psychiatry*, 78(4), 387-397. <https://doi.org/10.1001/jamapsychiatry.2020.4172>
- Stein, F., & Kircher, T. (2025). Transdiagnostic findings across major depressive disorder, bipolar disorder and schizophrenia: A qualitative review. *Journal of Affective Disorders*, 387, 119464. <https://doi.org/10.1016/j.jad.2025.119464>
- Ogasawara, K., Nakamura, Y., Kimura, H., Aleksic, B., & Ozaki, N. (2018). Issues on the diagnosis and etiopathogenesis of mood disorders: Reconsidering DSM-5. *Journal of Neural Transmission*, 125(2), 211-222. <https://doi.org/10.1007/s00702-017-1828-2>
- Watson, D., Levin-Aspensson, H. F., Waszczuk, M. A., Conway, C. C., Dalgleish, T., Dretsch, M. N., ... HiTOP Utility Workgroup (2022). Validity and utility of Hierarchical Taxonomy of Psychopathology (HiTOP): III. Emotional dysfunction superspectrum. *World psychiatry*, 21(1), 26–54. <https://doi.org/10.1002/wps.20943>

Week Beginning September 28: Fear/Avoidance and Habit Learning Mechanisms

Epidemiology and clinical presentation of anxiety disorders and OCD spectrum; fear conditioning, extinction, and intolerance of uncertainty; cognitive and attentional biases in anxiety; habit formation and error monitoring in OCD; cultural and developmental moderators of anxiety and OCD expression; transdiagnostic processes linking fear and compulsivity

- *Penninx, B. W., Pine, D. S., Holmes, E. A., & Reif, A. (2021). Anxiety disorders. *The Lancet*, 397(10277), 914-927. [https://doi.org/10.1016/S0140-6736\(21\)00359-7](https://doi.org/10.1016/S0140-6736(21)00359-7)
- Szuhany, K. L., & Simon, N. M. (2022). Anxiety disorders: A review. *JAMA*, 328(24), 2431-2445. <https://doi.org/10.1001/jama.2022.22744>
- *Craske, M. G., Hermans, D., & Vervliet, B. (2018). State-of-the-art and future directions for extinction as a translational model for fear and anxiety. *Philosophical Transactions of the Royal Society B: Biological Sciences*, 373(1742), 20170025. <https://doi.org/10.1098/rstb.2017.0025>
- Fullana, M. A., Tortella-Feliu, M., Fernández de la Cruz, L., et al. (2020). Risk and protective factors for anxiety and obsessive-compulsive disorders: An umbrella review of systematic reviews and meta-analyses. *Psychological Medicine*, 50(8), 1300-1315. <https://doi.org/10.1017/S0033291719001247>
- Cooper, S. E., & Dunsmoor, J. E. (2021). Fear conditioning and extinction in obsessive-compulsive disorder: A systematic review. *Neuroscience & Biobehavioral Reviews*, 129, 75-94. <https://doi.org/10.1016/j.neubiorev.2021.07.026>
- Kausche, F. M., Carsten, H. P., Sobania, K. M., & Riesel, A. (2025). Fear and safety learning in anxiety- and stress-related disorders: An updated meta-analysis. *Neuroscience & Biobehavioral Reviews*, 169, 105983. <https://doi.org/10.1016/j.neubiorev.2024.105983>
- Marin, M. F., Hammoud, M. Z., Klumpp, H., Simon, N. M., & Milad, M. R. (2020). Multimodal categorical and dimensional approaches to understanding threat conditioning and its extinction in individuals with anxiety disorders. *JAMA Psychiatry*, 77(6), 618-627. <https://doi.org/10.1001/jamapsychiatry.2019.4833>

- Milad, M. R., Furtak, S. C., Greenberg, J. L., et al. (2013). Deficits in conditioned fear extinction in obsessive-compulsive disorder and neurobiological changes in the fear circuit. *JAMA Psychiatry*, 70(6), 608-618. <https://doi.org/10.1001/jamapsychiatry.2013.914>
- Pittig, A., Treanor, M., LeBeau, R. T., & Craske, M. G. (2018). The role of associative fear and avoidance learning in anxiety disorders: Gaps and directions for future research. *Neuroscience & Biobehavioral Reviews*, 88, 117-140. <https://doi.org/10.1016/j.neubiorev.2018.03.015>
- Leung, C. J., Yiend, J., Trotta, A., & Lee, T. M. C. (2022). The combined cognitive bias hypothesis in anxiety: A systematic review and meta-analysis. *Journal of Anxiety Disorders*, 89, 102575. <https://doi.org/10.1016/j.janxdis.2022.102575>
- Lavigne, K. M., Deng, J., Raucher-Chéné, D., et al. (2024). Transdiagnostic cognitive biases in psychiatric disorders: A systematic review and network meta-analysis. *Progress in Neuro-Psychopharmacology & Biological Psychiatry*, 129, 110894. <https://doi.org/10.1016/j.pnpbp.2023.110894>
- American Psychiatric Association. (2022). Anxiety disorders. In *Diagnostic and Statistical Manual of Mental Disorders* (5th ed., text rev.; pp. 181-230). American Psychiatric Publishing.
- American Psychiatric Association. (2022). Obsessive-compulsive and related disorders. In *Diagnostic and Statistical Manual of Mental Disorders* (5th ed., text rev.; pp. 231-294). American Psychiatric Publishing.
- Dougherty, D. D., Brennan, B. P., Stewart, S. E., et al. (2018). Neuroscientifically informed formulation and treatment planning for patients with obsessive-compulsive disorder: A review. *JAMA Psychiatry*, 75(10), 1081-1087. <https://doi.org/10.1001/jamapsychiatry.2018.0930>
- Waters, A. M., & Craske, M. G. (2016). Towards a cognitive-learning formulation of youth anxiety: A narrative review of theory and evidence and implications for treatment. *Clinical Psychology Review*, 50, 50-66. <https://doi.org/10.1016/j.cpr.2016.09.008>

Week Beginning October 5: Environmental Adversity and Trauma as Transdiagnostic Risks

Epidemiology and course of PTSD and Stress-Related Disorders; dose/timing of trauma and cumulative adversity; biological stress systems and neural correlates (threat vs deprivation); dissociation and heterogeneity of trauma responses; minority stress, racism, and inequity in trauma exposure and outcomes; trauma's role as a cross-cutting mechanism in psychopathology

- *Afifi, T. O., Stewart-Tufescu, A., Taillieu, T., Salmon, S., Fortier, J., & Roos, J. (2022). Adverse childhood experiences. In G. Asmundson (Ed.) *Comprehensive Clinical Psychology* (2nd ed., Vol. 11, pp. 1-16). Elsevier. <https://dx.doi.org/10.1016/B978-0-12-818697-8.00038-8>
- *Racine, N., Hentges, R., McArthur, B. A., & Madigan, S. (2022). The intergenerational transmission of risk and psychopathology. In G. Asmundson (Ed.) *Comprehensive Clinical Psychology* (2nd ed., Vol. 11, pp. 17-31). Elsevier. <https://dx.doi.org/10.1016/B978-0-12-818697-8.00147-3>
- *Hawn, S. E., Wolf, E. J., Neale, Z., & Miller, M. W. (2022). Conceptualizing traumatic stress and the structure of posttraumatic psychopathology through the lenses of RDoC and HiTOP. *Clinical Psychology Review*, 95, 102177. <https://doi.org/10.1016/j.cpr.2022.102177>
- *Shalev, A., Liberzon, I., & Marmar, C. (2017). Post-traumatic stress disorder. *The New England Journal of Medicine*, 376(25), 2459-2469. <https://doi.org/10.1056/NEJMr1612499>
- Bryant, R. A. (2019). Post-traumatic stress disorder: A state-of-the-art review of evidence and challenges. *World Psychiatry*, 18(3), 259-269. <https://doi.org/10.1002/wps.20656>

- American Psychiatric Association. (2022). Trauma- and stressor-related disorders. In *Diagnostic and Statistical Manual of Mental Disorders* (5th ed., text rev.; pp. 295-328). American Psychiatric Publishing.
- Hamby, S., Elm, J. H. L., Howell, K. H., & Merrick, M. T. (2021). Recognizing the cumulative burden of childhood adversities transforms science and practice for trauma and resilience. *The American Psychologist*, 76(2), 230-242. <https://doi.org/10.1037/amp0000763>
- Cooke, E. M., Connolly, E. J., Boisvert, D. L., & Hayes, B. E. (2023). A systematic review of the biological correlates and consequences of childhood maltreatment and adverse childhood experiences. *Trauma, Violence, & Abuse*, 24(1), 156-173. <https://doi.org/10.1177/15248380211021613>
- Godoy, L. D., Rossignoli, M. T., Delfino-Pereira, P., Garcia-Cairasco, N., & de Lima Umeoka, E. H. (2018). A comprehensive overview on stress neurobiology: Basic concepts and clinical implications. *Frontiers in Behavioral Neuroscience*, 12, 127. <https://doi.org/10.3389/fnbeh.2018.00127>
- Lebois, L. A. M., Kumar, P., Palermo, C. A., et al. (2022). Deconstructing dissociation: A triple network model of trauma-related dissociation and its subtypes. *Neuropsychopharmacology*, 47(13), 2261-2270. <https://doi.org/10.1038/s41386-022-01468-1>
- Hogg, B., Gardoki-Souto, I., Valiente-Gómez, A., et al. (2023). Psychological trauma as a transdiagnostic risk factor for mental disorder: An umbrella meta-analysis. *European Archives of Psychiatry and Clinical Neuroscience*, 273(2), 397-410. <https://doi.org/10.1007/s00406-022-01495-5>
- Gershuny, B. S., & Thayer, J. F. (1999). Relations among psychological trauma, dissociative phenomena, and trauma-related distress: A review and integration. *Clinical Psychology Review*, 19(5), 631-657. [https://doi.org/10.1016/S0272-7358\(98\)00103-2](https://doi.org/10.1016/S0272-7358(98)00103-2)

Week Beginning October 12: Psychosis Disorders and Cognitive/Affective Dysfunction

Epidemiology, prodrome, and course of psychotic disorders; dopamine, glutamate, and prediction error models; neurocognitive deficits and social cognition impairments; environmental risks: cannabis, obstetric complications, urbanicity, racism; genetic contributions and gene-environment interplay; dimensional models of psychosis

- *Jonas, K. G., et al. (2024). The HiTOP psychosis superspectrum: Integrating nosology, etiology, development, genetic structure, environmental risk factors, and course into a dimensional model. *Psychological Medicine*. Advance online publication. <https://doi.org/10.1017/S0033291724001234>
- *Howes, O. D., & Murray, R. M. (2014). Schizophrenia: An integrated sociodevelopmental-cognitive model. *The Lancet*, 383(9929), 1677-1687. [https://doi.org/10.1016/S0140-6736\(13\)62036-X](https://doi.org/10.1016/S0140-6736(13)62036-X)
- Murray, R. M., et al. (2020). The GAP study: Genetic and environmental determinants of the epidemiology, prodrome, course, and outcome of psychosis. *Schizophrenia Bulletin*, 46(1), 1-13. <https://doi.org/10.1093/schbul/sbz120>
- Lin, B. D., et al. (2022). Exposome-wide analysis of environmental risks for psychotic experiences: Evidence from UK Biobank and Mendelian randomization. *Molecular Psychiatry*, 27(9), 3774-3785. <https://doi.org/10.1038/s41380-022-01567-3>
- Radua, J., et al. (2018). What causes psychosis? An umbrella review of risk and protective factors. *World Psychiatry*, 17(1), 49-66. <https://doi.org/10.1002/wps.20490>

- Stilo, S. A., & Murray, R. M. (2010). The epidemiology of schizophrenia: Replacing dogma with knowledge. *Dialogues in Clinical Neuroscience*, 12(3), 305-315. <https://doi.org/10.31887/DCNS.2010.12.3/sstilo>
- Oliver, D., et al. (2024). Environmental risk factors for psychosis: Mechanistic links, gene–environment interactions, and preventive opportunities. *Nature Reviews Psychiatry*. Advance online publication. <https://doi.org/10.1038/s44159-024-00112-9>
- Zwicker, A., et al. (2018). Gene–environment interplay in psychosis: The role of polygenic risk, cannabis use, and specific gene–environment interactions. *Schizophrenia Research*, 202, 14-23. <https://doi.org/10.1016/j.schres.2018.06.021>
- Davis, J., et al. (2016). Beyond the two-hit hypothesis of schizophrenia. *Translational Psychiatry*, 6(7), e735. <https://doi.org/10.1038/tp.2015.239>
- Thomas, M., Rakesh, D., Whittle, S., et al. (2023). The neural, stress hormone and inflammatory correlates of childhood deprivation and threat in psychosis: A systematic review. *Psychoneuroendocrinology*, 157, 106371. <https://doi.org/10.1016/j.psyneuen.2023.106371>
- American Psychiatric Association. (2022). Schizophrenia spectrum and other psychotic disorders. In *Diagnostic and Statistical Manual of Mental Disorders* (5th ed., text rev.; pp. 101-122). American Psychiatric Publishing.

Week Beginning October 19: Disinhibition and Reward

Epidemiology and developmental course of externalizing disorders (ADHD, conduct, antisocial, SUDs); mechanisms (reinforcement learning, delay discounting, incentive salience, executive control)

- *Beauchaine, T. P., Zisner, A. R., & Sauder, C. L. (2017). Trait impulsivity and the externalizing spectrum. *Annual Review of Clinical Psychology*, 13, 343-368. <https://doi.org/10.1146/annurev-clinpsy-021815-093253>
- *Krueger, R. F., Hobbs, K. A., Conway, C. C., et al. (2021). Validity and utility of hierarchical taxonomy of psychopathology (HiTOP): II. Externalizing superspectrum. *World Psychiatry*, 20(2), 171-193. <https://doi.org/10.1002/wps.20844>
- *Hyde, L. W., Trentacosta, C. J., & Bezek, J. L. (2025). Annual research review: Early conduct problems—Precursors, outcomes, and etiology. *Journal of Child Psychology and Psychiatry*. Advance online publication. <https://doi.org/10.1111/jcpp.70031>
- American Psychiatric Association. (2022). Disruptive, impulse-control, and conduct disorders. In *Diagnostic and Statistical Manual of Mental Disorders* (5th ed., text rev.; pp. 461–484). American Psychiatric Publishing.
- Retz, W., Ginsberg, Y., Turner, D., et al. (2021). Attention-deficit/hyperactivity disorder (ADHD), antisociality and delinquent behavior over the lifespan. *Neuroscience & Biobehavioral Reviews*, 120, 236-248. <https://doi.org/10.1016/j.neubiorev.2020.11.025>
- Bell, Z. E., Fristad, M. A., Youngstrom, E. A., Arnold, L. E., & Beauchaine, T. P. (2022). Attention-deficit/hyperactivity disorder symptoms and externalizing progression in the LAMS study: A test of trait impulsivity theory. *Journal of the American Academy of Child & Adolescent Psychiatry*, 61(2), 298-307. <https://doi.org/10.1016/j.jaac.2021.05.018>
- Martinez, J. L., Hasty, C., Morabito, D., et al. (2022). Perceptions of childhood unpredictability, delay discounting, risk-taking, and adult externalizing behaviors: A life-history approach. *Development and Psychopathology*, 34(2), 705-717. <https://doi.org/10.1017/S0954579421001607>

- Colaizzi, J. M., Fligel, S. B., Gearhardt, A. N., et al. (2023). The propensity to sign-track is associated with externalizing behavior and distinct patterns of reward-related brain activation in youth. *Scientific Reports*, 13(1), 4402. <https://doi.org/10.1038/s41598-023-30906-3>
- American Psychiatric Association. (2022). Substance-related and addictive disorders. In *Diagnostic and Statistical Manual of Mental Disorders* (5th ed., text rev.; pp. 485-556). American Psychiatric Publishing.
- Crum, K. I., Aloji, J., Blair, K. S., et al. (2023). Latent profiles of substance use, early life stress, and attention/externalizing problems and their association with neural correlates of reinforcement learning in adolescents. *Psychological Medicine*, 53(15), 7358-7367. <https://doi.org/10.1017/S0033291723000971>

Week Beginning October 26: Interpersonal Dysfunction

Epidemiology and dimensional conceptualizations of personality pathology; alternative DSM-5 model for personality disorders, trait-based approaches; intersection of externalizing and personality dysfunction with environmental adversity and inequities; integration of externalizing and personality pathology in transdiagnostic models

- American Psychiatric Association. (2022). Personality disorders. In *Diagnostic and Statistical Manual of Mental Disorders* (5th ed., text rev.; pp. 727-776). American Psychiatric Publishing.
- Sharp, C. (2022). Personality disorders. *The New England Journal of Medicine*, 387(10), 916-923. <https://doi.org/10.1056/NEJMra2120164>
- *Crişan, Ş., & Nechita, D. (2025). Personality disorders and their association with adverse childhood events. *Current Opinion in Psychiatry*, 38(1), 59-65. <https://doi.org/10.1097/YCO.0000000000000959>
- Russell, J. D., Heyn, S. A., Peverill, M., DiMaio, S., & Herringa, R. J. (2025). Traumatic and adverse childhood experiences and developmental differences in psychiatric risk. *JAMA Psychiatry*, 82(1), 66-74. <https://doi.org/10.1001/jamapsychiatry.2024.3231>
- *Rodriguez-Seijas, C., Eaton, N. R., & Krueger, R. F. (2015). How transdiagnostic factors of personality and psychopathology can inform clinical assessment and intervention. *Journal of Personality Assessment*, 97(5), 425-435. <https://doi.org/10.1080/00223891.2015.1055752>

Week Beginning November 2: Integrative Nosology I: Taxonomy vs. Nosology

Historical and conceptual evolution of classification; taxonomy distinctions; critiques of categorical models; DSM vs ICD organization, strengths, limitations; global mental health considerations and equity in classification

- *Berenbaum, H. (2013). Classification and psychopathology research. *Journal of Abnormal Psychology*, 122(3), 894-901. <https://doi.org/10.1037/a0033096>
- *Schultze-Lutter, F., Schmidt, S. J., & Theodoridou, A. (2018). Psychopathology—A precision tool in need of re-sharpening. *Frontiers in Psychiatry*, 9, 446. <https://doi.org/10.3389/fpsy.2018.00446>
- *McKay, D. & Pedowitz, A. (2022). Utility of analog samples in psychopathology research. In G. Asmundson (Ed.) *Comprehensive Clinical Psychology* (2nd ed., Vol. 3, pp. 104-108). Elsevier. <https://dx.doi.org/10.1016/B978-0-12-818697-8.00237-5>

- Ruscio, J. & Wang, S. (2022). Taxometric analysis. In G. Asmundson (Ed.) *Comprehensive Clinical Psychology* (2nd ed., Vol. 3, pp. 148-175). Elsevier. <https://dx.doi.org/10.1016/B978-0-12-818697-8.00217-X>
- Aftab, A., & Ryznar, E. (2021). Conceptual and historical evolution of psychiatric nosology. *International Review of Psychiatry*, 33(5), 486-499. <https://doi.org/10.1080/09540261.2020.1828306>
- *Leucht, S., van Os, J., Jäger, M., & Davis, J. M. (2024). Prioritization of psychopathological symptoms and clinical characterization in psychiatric diagnoses: A narrative review. *JAMA Psychiatry*, 81(11), 1149-1158. <https://doi.org/10.1001/jamapsychiatry.2024.2652>
- *First, M. B., Gaebel, W., Maj, M., Reed, G. M., & Saxena, S. (2021). An organization- and category-level comparison of diagnostic requirements for mental disorders in ICD-11 and DSM-5. *World Psychiatry*, 20(1), 34-51. <https://doi.org/10.1002/wps.20825>
- Widiger, T. A., & Crego, C. (2018). Mental disorders as discrete clinical conditions: Dimensional versus categorical classification (pp. 3-7). In D. C. Beidel & B. C. Frueh (Eds.), *Adult Psychopathology and Diagnosis* (8th ed.). Wiley.
- Bach, B., Bo, S., & Keeley, J. W. (2022). *Diagnostic systems and models: DSM-5 Criteria, ICD-11 Guidelines, and Dimensional Horizons*. In G. Asmundson (Ed.), *Comprehensive Clinical Psychology* (2nd ed., Vol. 10, pp. 307-320). Elsevier. <https://www.sciencedirect.com/science/article/pii/B9780128186978001114?via%3Dihub>
- Fluyau, D. (2018). Integrating DSM/ICD, research domain criteria, and descriptive psychopathology in teaching and practice of psychiatry. *Frontiers in Psychiatry*, 9, 484. <https://doi.org/10.3389/fpsy.2018.00484>

Week Beginning November 9: Integrative Nosology II: Dimensional Models

Core dimensional frameworks; evidence base for hierarchical and quantitative classification models; integrating dimensional and categorical systems; linking dimensions to mechanisms, biomarkers, and transdiagnostic processes; emerging directions and debates in dimensional nosology

- *Lahey, B. B., Moore, T. M., Kaczurkin, A. N., & Zald, D. H. (2021). Hierarchical models of psychopathology: Empirical support, implications, and remaining issues. *World Psychiatry*, 20(1), 57-63. <https://doi.org/10.1002/wps.20824>
- *Krueger, R. F., Kotov, R., Watson, D., et al. (2018). Progress in achieving quantitative classification of psychopathology. *World Psychiatry*, 17(3), 282-293. <https://doi.org/10.1002/wps.20566>
- *Sanislow, C. A., Pine, D. S., Quinn, K. J., Kozak, M. J., Garvey, M. A., Heinssen, R. K., ... & Cuthbert, B. N. (2010). Developing constructs for psychopathology research: Research domain criteria. *Journal of Abnormal Psychology*, 119(4), 631-639. <https://doi.org/10.1037/a0020909>
- Kotov, R., Krueger, R. F., Watson, D., Achenbach, T. M., Althoff, R. R., Bagby, R. M., ... Zimmermann, J. (2017). The Hierarchical Taxonomy of Psychopathology (HiTOP): A dimensional alternative to traditional nosologies. *Journal of Abnormal Psychology*, 126(4), 454-477. <https://doi.org/10.1037/abn0000258>
- Michelini, G., Palumbo, I. M., DeYoung, C. G., Latzman, R. D., & Kotov, R. (2021). Linking RDoC and HiTOP: A new interface for advancing psychiatric nosology and neuroscience. *Clinical Psychology Review*, 86, 102025. <https://doi.org/10.1016/j.cpr.2021.102025>

- Cicero, D. C., Ruggero, C. J., Balling, C. E., Kotov, R., ... Wright, A. G. C. (2024). State of the science: The Hierarchical Taxonomy of Psychopathology (HiTOP). *Behavior Therapy*, 55(6), 1114-1129. <https://doi.org/10.1016/j.beth.2024.05.001>

Week Beginning November 16: Comorbidity & Heterogeneity in Psychopathology

Comorbidity patterns across internalizing/externalizing/psychosis; heterogeneity within diagnostic categories; network approaches and the “p factor.”

- *Caspi A., Houts, R.M., Belsky, D.W., et al. (2014). The p Factor: One General Psychopathology Factor in the Structure of Psychiatric Disorders? *Clinical Psychological Science*, 2(2): 119-137. doi:10.1177/2167702613497473
- *Blanchard, M. A. & Heeren, A. (2022). Ongoing and future challenges of the network approach to psychopathology: From theoretical conjectures to clinical translations. In G. Asmundson (Ed.) *Comprehensive Clinical Psychology* (2nd ed., Vol. 11, pp. 32-46). Elsevier. <https://dx.doi.org/10.1016/B978-0-12-818697-8.00044-3>
- *Robinaugh, D. J., Hoekstra, R. H. A., Toner, E. R., & Borsboom, D. (2020). The network approach to psychopathology: A review of the literature 2008–2018 and an agenda for future research. *Psychological Medicine*, 50(3), 353-366. <https://doi.org/10.1017/S0033291719003404>
- [Re-review] Feczko, E., Miranda-Dominguez, O., Marr, M., Graham, A. M., Nigg, J. T., & Fair, D. A. (2019). The heterogeneity problem: approaches to identify psychiatric subtypes. *Trends in Cognitive Sciences*, 23(7), 584-601.
- [Re-review] Caspi A, Houts RM, Ambler A, et al. Longitudinal Assessment of Mental Health Disorders and Comorbidities Across 4 Decades Among Participants in the Dunedin Birth Cohort Study. *JAMA Netw Open*. 2020;3(4):e203221. doi:10.1001/jamanetworkopen.2020.3221

Week Beginning November 23: No Class

Fall Break and Thanksgiving Holidays (Nov. 23–29)

Week Beginning November 30: Integrative Psychopathology Analysis

INTEGRATIVE PSYCHOPATHOLOGY ANALYSIS ORAL PRESENTATIONS

Week Beginning December 7: Integration Across Domains

Synthesizing across disorder domains studied this semester; integrating developmental and cultural considerations into psychopathology science; addressing comorbidity, heterogeneity, and nosological challenges

- Allwood, M. A., Ford, J. D., & Levendosky, A. (2021). Introduction to the special issue: Disproportionate trauma, stress, and adversities as a pathway to health disparities among disenfranchised groups globally. *Journal of Traumatic Stress*, 34(5), 899-904. <https://doi.org/10.1002/jts.22743>
- Comas-Díaz, L., Hall, G. N., & Neville, H. A. (2019). Racial trauma: Theory, research, and healing: Introduction to the special issue. *The American Psychologist*, 74(1), 1-5. <https://doi.org/10.1037/amp0000442>

- Kirkinis, K., Pieterse, A. L., Martin, C., Agiliga, A., & Brownell, A. (2021). Racism, racial discrimination, and trauma: A systematic review of the social science literature. *Ethnicity & Health, 26*(3), 392-412. <https://doi.org/10.1080/13557858.2018.1514453>
- Powell, W. (2021). Commentary on the special issue on disproportionate exposure to trauma: Disrupting single stories, broadening the aperture, and embracing the trauma socioexposome in research with minoritized individuals. *Journal of Traumatic Stress, 34*(5), 1056-1060. <https://doi.org/10.1002/jts.22747>

Required In Class Activity:

Description: This session serves as the capstone for PSYC 7308, requiring students to demonstrate independent synthesis at the doctoral level. No advance preparation is expected; students should not attempt to pre-prepare or cram for this class. The purpose is to demonstrate mastery with real-time facility, showing the ability to draw on and integrate what has been learned across the semester without scripted notes.

After submitting the Integrative Psychopathology Analysis paper at the start of class, students will work in self-selected small groups to integrate findings from epidemiology, genetics, psychophysiology, cognition/affect, and environmental factors across major disorder domains, explicitly considering developmental trajectories, cultural/contextual influences, and patterns of comorbidity. Each group will: (1) identify at least one theme or mechanism that cuts across two or more disorder domains, (2) analyze how developmental or cultural/contextual factors shape that theme, and (3) highlight at least one unresolved challenge for psychopathology science (e.g., comorbidity, heterogeneity, classification limits). Groups will have 40 minutes to prepare their synthesis, followed by ~5-minute presentations. As presentations unfold, the class will collaboratively construct a shared integration map capturing cross-cutting themes, developmental and cultural considerations, and unresolved challenges. The instructor's role will be limited to facilitating time and logistics, as the goal is for students to demonstrate scholarly independence and the capacity to integrate psychopathology research at a level expected of doctoral clinical scientists.

Assignments and Exams

Integrative Psychopathology Analysis - Paper: This major assignment requires critically integrating nosological systems, empirical findings, and ethical considerations. Select the domain of psychopathology most salient to your faculty mentor's area of research. Obtain instructor approval of the selected domain to ensure it is neither too narrow nor too broad. Prepare a 15 to 20-page paper (excluding title page, abstract, and references) in APA style that accomplishes following:

- Review the empirical literature on the selected domain, including epidemiology, cognitive and affective correlates, genetic and neurobiological findings, and environmental/contextual risk factors.

- Apply multiple nosological frameworks (e.g., DSM, ICD, HiTOP, RDoC) to the domain, analyzing convergences, divergences, and implications for case conceptualization and research design.
- Evaluate methodological strengths and weaknesses in the existing literature, including issues of construct validity, measurement, and generalizability.
- Integrate ethical principles into the analysis, considering diagnostic labeling, cultural applicability, equity in care, and how diagnostic practices may contribute to or mitigate stigma and disparities.
- Conclude with recommendations for how future nosological systems and ethical guidelines could evolve together to better serve both science and practice.

Students are expected to draw on at least 20 primary-source empirical references, with an emphasis on recent scholarship (i.e., papers published within the preceding 10 years). Papers are due December 7th and should be submitted as a PDF.

Papers will be evaluated on a 70-point scale as follows:

Integration and comparison of nosological frameworks (20 points)
Synthesis of psychopathology research across epidemiology, biology, cognition/affect, and environment (20 points)
Methodological critique and analysis of literature quality (15 points)
Integration of ethical principles and cultural considerations (10 points)
Clarity, organization, originality, and APA adherence (5 points)

Integrative Psychopathology Analysis - Oral Presentation: In addition to the written paper, students will deliver a 20-minute professional presentation of their analysis, followed by a brief Q&A. The presentation must highlight the key findings of the literature review, integrate multiple nosological frameworks, and articulate the ethical implications of classification and diagnosis. This requirement is intended to scaffold skills needed for proposal defenses, professional conferences, and future academic job talks. Students are expected to prepare and deliver their presentation at a professional standard, including appropriate attire and the use of visual aids (e.g., slides, figures) that meet the expectations of academic and scientific audiences.

Presentations must be completed within the 20-minute limit while adequately covering all major required content areas: literature synthesis, nosological integration, methodological critique, ethical considerations, and future directions. Presentations that exceed 20 minutes will be penalized at a rate of 5 points per additional minute, up to a maximum of 15 points. Presentations that omit a required content area (in an effort to end within 20 minutes) will not only lose the points associated with that criterion but will also trigger an additional coverage penalty of up to 15 points, reflecting the professional expectation that all essential domains must be addressed within the allotted time.

Presentations will be evaluated on a 30-point scale as follows:

Effective synthesis and prioritization of key material within the 20-minute limit (10 points)

Integration and comparison of nosological frameworks (5 points)
Coverage of empirical findings across epidemiology, biology, cognition/affect, and environment (5 points)
Integration of ethical principles and cultural considerations (5 points)
Clarity, organization, professionalism, and engagement during presentation (5 points)

Penalties for exceeding time or failing to cover required domains will be deducted from the total score.

Final Exam: The course concludes with a comprehensive take-home final examination. This exam is designed to assess each student's ability to integrate concepts from across the semester, including psychopathology theory, diagnostic systems, genetic and biological correlates, epidemiology, cognitive and affective mechanisms, and environmental risk factors. The exam will consist of essay-style questions and applied case analyses that require students to synthesize empirical literature, evaluate methodological rigor, and justify diagnostic and conceptual choices within a clinical science framework. Students are expected to demonstrate independent critical thinking and scholarly writing at a level consistent with doctoral training. The final exam is cumulative, closed to collaboration, and must be completed within the assigned time frame. The exam will open Dec 4th and is due Dec 14th.

Grading

Final grades will be determined by weighting the earned scores on the assignments and exams as follows:

Requirement	Weight in Final Grade	Letter	Percentage	Letter	Percentage
Preparation & Participation	20%	A	93-100	C	73-76
Integrative Psychopathology Analysis - Paper	35%	A-	90-92	C-	70-72
Integrative Psychopathology Analysis - Presentation	15%	B+	87-89	D+	67-69
Final Exam	30%	B	83-86	D	63-66
		B-	80-82	D-	60-62
		C+	77-79	F	59 or lower

All assigned grades will adhere to the allocations above and will not be changed upon request. Any grade dispute must be raised within one week of the posting of the grade in question. Grades will be posted on eLearning.

Course Policies

Incompletes: Incompletes will only be given if the only remaining requirement is completion of the final exam. An incomplete grade would need to be arranged prior to the final exam and would require approval of the instructor based upon a substantial excuse.

Extra Credit: There is no extra credit in this course.

Late Work: A penalty of one letter grade will be assessed for each calendar day late.

Communication

This course utilizes online tools for interaction and communication. Some external communication tools such as regular email and a web conferencing tool may also be used during the semester. For more details, please visit the [Student eLearning Tutorials](#) webpage for video demonstrations on eLearning tools. Please use email or office hours to ask questions of the instructor. Student emails will be answered within 3 working days under normal circumstances.

Class Materials

The instructor may provide class materials that will be made available to all students registered for this class as they are intended to supplement the classroom experience. These materials may be downloaded during the course; however, these materials are for registered students' use only. Class materials may not be reproduced or shared with those not in class or uploaded to other online environments except to implement an approved AccessAbility Resource Center accommodation. Failure to comply with these University requirements is a violation of the [Student Code of Conduct](#).

Class Attendance

Regular and punctual class attendance is expected. Students who fail to attend class regularly are inviting scholastic difficulty.

Class Participation

Regular class participation is expected. Students who fail to participate in class regularly are inviting scholastic difficulty. A portion of the grade for this course is directly tied to your participation in this class. It also includes engaging in group or other activities during class that solicit your feedback on homework assignments, readings, or materials covered in the lectures (and/or labs). Class participation is documented by faculty. Successful participation is defined as consistently adhering to University requirements, as presented in this syllabus. Failure to comply with these University requirements is a violation of the [Student Code of Conduct](#).

Texas Senate Bill 17, the recent law that prohibits diversity, equity, and inclusion programs and activities at public universities in Texas, does not in any way apply to academic course instruction. Students should not feel the need to self-censor or limit their participation in academic courses pertaining to topics of race and racism, structural inequality, LGBTQ+ issues, or diversity, equity, and inclusion, and related topics.

Class Recordings

Students are expected to follow appropriate University policies and maintain the security of passwords used to access recorded lectures. Unless the AccessAbility Resource Center has approved the student to record the instruction, students are expressly prohibited from recording any part of this course. Recordings may not be published, reproduced, or shared with those not in the class, or uploaded to other online environments except to implement an

approved AccessAbility Resource Center accommodation. Failure to comply with these University requirements is a violation of the [Student Code of Conduct](#).

The instructor may record meetings of this course. These recordings will be made available to all students registered for this class if the intent is to supplement the classroom experience. If the instructor or a UTD school/department/office plans any other uses for the recordings, consent of the students identifiable in the recordings is required prior to such use unless an exception is allowed by law.

Academic Integrity / Misconduct

Students are reminded that academic misconduct will not be tolerated. For example: (a) copying another student's work, in part or in whole; (b) unauthorized communication to obtain information about a test(s); (c) using unauthorized material during a test; (d) handing in any work product that is not your own; (e) signing someone else's name; (f) failing to give credit to the intellectual property of another in any written work (including, but not limited to, plagiarizing). For a more extensive listing of behaviors that are considered misconduct please refer to the UT Dallas [Community Standards and Conduct](#).

Comet Creed

This creed was voted on by the UT Dallas student body in 2014. It is a standard that Comets choose to live by and encourage others to do the same:

"As a Comet, I pledge honesty, integrity, and service in all that I do."

Accommodations for Students with Disabilities

Please review [the section](#) within the UT Dallas Syllabus Policies and Procedures webpage.

Server Unavailability or Other Technical Difficulties

The University is committed to providing a reliable learning management system to all users. However, in the event of any unexpected server outage or any unusual technical difficulty which prevents students from completing a time sensitive assessment activity, the instructor will provide an appropriate accommodation based on the situation. Students should immediately report any problems to the instructor and contact the online [eLearning Help Desk](#). The instructor and the eLearning Help Desk will work with the student to resolve any issues at the earliest possible time.

Academic Support Resources

Please visit the [Academic Support Resources](#) page to view the University's academic support resources for all students.

UT Dallas Syllabus Policies and Procedures

The information contained in the following link constitutes the University's policies and procedures segment of the course syllabus. Please review the catalog sections regarding the [credit/no credit](#) or [pass/fail](#) grading option and withdrawal from class. Please go to [UT Dallas Syllabus Policies](#) webpage for these policies.

CPSY 7300: Clinical Psychological Science I
Clinical Science Foundations: Assessment, Mechanisms, and Translation

Course Pre-requisites, Co-requisites, and/or Other Restrictions

PSYC 7308. Department consent required.

Course Description

This course provides foundational training in the clinical science of assessment, with emphasis on measurement theory, psychometric methods, and the integration of assessment into research and practice. Students critically examine the empirical literature on validity, reliability, structured interviewing, cognitive and achievement testing, self-report measures, and fidelity checks, alongside advanced topics such as performance validity testing, ecological validity, and the reliability paradox. The course also introduces mechanism science through the study of mediators and moderators of change and connects assessment to routine outcome monitoring and introductory dissemination and implementation (D&I) research. Ethical, developmental, and multicultural considerations are woven throughout. Major assignments include a psychometrics project, requiring students to evaluate the reliability, validity, and cultural fairness of a standardized measure, along with cumulative assessments that test integrative knowledge of assessment science.

Student Learning Objectives/Outcomes

The learning activities assigned across this course are intended to develop critical thinking skills essential for evaluating psychological research and practice. Students will be able to:

- Critically evaluate psychometric evidence for major assessment tools, including issues of reliability, validity, invariance, and ecological utility.
- Apply advanced measurement and design methods (e.g., factor analysis, IRT, mediation/moderation models) to the evaluation of psychopathology constructs.
- Integrate epidemiological, biological, cognitive/affective, and environmental perspectives when interpreting assessment data.
- Analyze the role of mediators and moderators in linking assessment findings to mechanisms of change in clinical science.
- Demonstrate competence in translating assessment science into practice through the completion of a Psychometrics Project that evaluates a standardized measure for clinical research use.
- Synthesize multicultural and ethical considerations into all aspects of assessment and measurement, including test selection, interpretation, and communication of results.

Successful completion of this course is expected to expand competency (consult the profession-wide competencies codebook for descriptions and behavioral anchors) in the following areas:

- Scientific Mindedness
- Scientific Foundations
- Knowledge of Ethical, Legal and Professional Standards and Guidelines
- Awareness and Application of Ethical Decision Making
- Empowerment

- Diagnosis
- Measurement and Psychometrics
- Application of Assessment Methods
- Diagnostic Conceptualization and Recommendations
- Intervention Planning
- Scientific Foundation of Professional Practice
- Treatment Monitoring
- Professional Identity
- Accountability
- Interpersonal Relationships
- Expressive Skills
- Affective Skills
- Supervision Skills Development
- Understands How Participation in Interdisciplinary Collaboration/ Consultation Enhances Outcomes

Course Access and Navigation

This course can be accessed using your UT Dallas NetID account on the [eLearning](#) website. Please see the course access and navigation section of the [Getting Started with eLearning](#) webpage for more information. To become familiar with the eLearning tool, please see the [Student eLearning Tutorials](#) webpage. UT Dallas provides eLearning technical support 24 hours a day, 7 days a week. The [eLearning Support Center](#) includes a toll-free telephone number for immediate assistance (1-866-588-3192), email request service, and an online chat service.

Course Schedule & Readings

The weekly reading lists in CPSY 7300 are intentionally expansive to reflect the breadth of salient scholarship. Each week, the instructor will designate a subset of core readings as required anchors for discussion, while the broader list functions as a curated library that students are expected to draw from when preparing their psychometrics project and final exam. All readings are provided on the eLearning course site.

Spring 2027 Weekly Schedule

Week Beginning January 18: Introduction to Clinical Psychological Science

Philosophy of science, pseudoscience, evidence hierarchies

- McFall, R.M. (1991). Manifesto for a science of clinical psychology. *The Clinical Psychologist*, 44, 75-91.
- Callahan, J. L. & Braggs, P. (2022). Doctoral training: Preparing for professional practice. In G. Asmundson (Ed.) *Comprehensive Clinical Psychology* (2nd ed., Vol. 2, pp. 55-65). Elsevier. <https://dx.doi.org/10.1016/B978-0-12-818697-8.00100-X>
- *Platt, J. R. (1964). Strong inference. *Science*, 146(3642), 347–353. <https://doi.org/10.1126/science.146.3642.347>
- Meehl, P. E. (1967). Theory-testing in psychology and physics: A methodological paradox. *Philosophy of Science*, 34(2), 103–115. <https://doi.org/10.1086/288135>
- *Meehl, P. E. (1978). Theoretical risks and tabular asterisks: Sir Karl, Sir Ronald, and the slow progress of soft psychology. *Journal of Consulting and Clinical Psychology*, 46(4), 806–834. <https://doi.org/10.1037/0022-006X.46.4.806>

- *Lilienfeld, S. O. (1998). Pseudoscience in contemporary clinical psychology: What it is and what we can do about it. *Clinical Psychology: Science and Practice*, 5(1), 3–29. <https://doi.org/10.1111/j.1468-2850.1998.tb00144.x>
- Ioannidis, J. P. A. (2005). Why most published research findings are false. *PLoS Medicine*, 2(8), e124. <https://doi.org/10.1371/journal.pmed.0020124>
- Sackett, D. L., Rosenberg, W. M. C., Gray, J. A. M., Haynes, R. B., & Richardson, W. S. (1996). Evidence based medicine: What it is and what it isn't. *BMJ*, 312(7023), 71–72. <https://doi.org/10.1136/bmj.312.7023.71>

Week Beginning January 25: Theory & Construct Validity in Clinical Science

Building, testing, and refining constructs; classic and modern perspectives

- *Cronbach, L. J., & Meehl, P. E. (1955). Construct validity in psychological tests. *Psychological Bulletin*, 52(4), 281–302. <https://doi.org/10.1037/h0040957>
- *Borsboom, D., Mellenbergh, G. J., & van Heerden, J. (2004). The concept of validity. *Psychological Review*, 111(4), 1061–1071. <https://doi.org/10.1037/0033-295X.111.4.1061>
- Kane, M. T. (2013). Validating the interpretations and uses of test scores. *Journal of Educational Measurement*, 50(1), 1–73. <https://doi.org/10.1111/jedm.12000>
- Bollen, K. A., & Lennox, R. (1991). Conventional wisdom on measurement: A structural equation perspective. *Psychological Bulletin*, 110(2), 305–314. <https://doi.org/10.1037/0033-2909.110.2.305>
- Edwards, J. R. (2003). Construct validation in organizational behavior research: Issues and techniques. *Organizational Research Methods*, 6(4), 327–351. <https://doi.org/10.1177/1094428103251549>
- *Flake, J. K., & Fried, E. I. (2020). Measurement schmeasurement: Questionable measurement practices and how to avoid them. *Advances in Methods and Practices in Psychological Science*, 3(4), 456–465. <https://doi.org/10.1177/2515245920952393>

Week Beginning February 1: Measurement-Based Care (MBC)

Routine outcome monitoring, ROM science, and ethical integration

- *Lewis, C. C., Boyd, M., Puspitasari, A., Navarro, E., Howard, J., Kassab, H., Hoffman, M., Scott, K., Lyon, A., Douglas, S., Simon, G., & Kroenke, K. (2019). Implementing measurement-based care in behavioral health: A review. *JAMA Psychiatry*, 76(3), 324–335. <https://doi.org/10.1001/jamapsychiatry.2018.3329>
- *Scott, K., & Lewis, C. C. (2015). Using measurement-based care to enhance any treatment. *Cognitive and Behavioral Practice*, 22(1), 49–59. <https://doi.org/10.1016/j.cbpra.2014.01.010>
- Lutz, W., Vehlen, A., & Schwartz, B. (2024). Data-informed psychological therapy, measurement-based care, and precision mental health. *Journal of Consulting and Clinical Psychology*, 92(10), 671–673. <https://doi.org/10.1037/ccp0000904>
- Fortney, J. C., Unützer, J., Wrenn, G., Pyne, J. M., Smith, G. R., Schoenbaum, M., & Harbin, H. T. (2017). A tipping point for measurement-based care. *Psychiatric Services*, 68(2), 179–188. <https://doi.org/10.1176/appi.ps.201500439>
- *Muir, H. J., Coyne, A. E., Morrison, N. R., Boswell, J. F., & Constantino, M. J. (2019). Ethical implications of routine outcomes monitoring for patients, psychotherapists, and mental health care systems. *Psychotherapy*, 56(4), 459–469. <https://doi.org/10.1037/pst0000246>

- Boswell, J. F., Kraus, D. R., Miller, S. D., & Lambert, M. J. (2015). Implementing routine outcome monitoring in clinical practice: Benefits, challenges, and solutions. *Psychotherapy*, 52(4), 411–421. <https://doi.org/10.1037/pst0000027>
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Week Beginning February 8: Research–Practice Integration in Assessment

Ecological validity, the reliability paradox, challenges in small effects

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Week Beginning February 15: Psychometric Principles & Advanced Methods

Reliability estimation, factor analysis, IRT, measurement of invariance

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Week Beginning February 22: Mediators & Moderators of Change

Conceptual/statistical models, moderators of treatment/assessment, mechanisms of change

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Week Beginning March 1: Structured & Semi-Structured Interview Methods

Psychometric properties and fidelity in structured interviewing

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Week Beginning March 8: Intelligence & Achievement Testing (Part 1)

History, structure, applications

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- Ritchie, S. J., & Tucker-Drob, E. M. (2018). How much does education improve intelligence? A meta-analysis. *Psychological Science*, 29(8), 1358-1369. <https://doi.org/10.1177/0956797618774253>
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Week Beginning March 15: Spring Break

Week Beginning March 22: Intelligence & Achievement Testing (Part 2)

Critical evaluation, biases, limitations

- Schaughency, E., Clifford, A. E., Carroll, J. L. D. (2022). Assessment of achievement and learning disabilities. In G. Asmundson (Ed.) *Comprehensive Clinical Psychology* (2nd ed., Vol. 4, pp. 141-166). Elsevier. <https://dx.doi.org/10.1016/B978-0-12-818697-8.00100-X>
- *Neisser, U., Boodoo, G., Bouchard, T. J., Boykin, A. W., Brody, N., Ceci, S. J., ... Urbina, S. (1996). Intelligence: Knowns and unknowns. *American Psychologist*, 51(2), 77–101.
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- Weiss, L. G., & Saklofske, D. H. (2020). Mediators of IQ test score differences across racial and ethnic groups: The case for environmental and social justice. *Personality and Individual Differences*, 161, 109962.

Week Beginning March 29: Personality and Self-Report Measures

Trait models, incremental validity, embedded scales

- *McCrae, R. R., & Costa, P. T. (1997). Personality trait structure as a human universal. *American Psychologist*, 52(5), 509–516. <https://doi.org/10.1037/0003-066X.52.5.509>
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Week Beginning April 5: Fidelity & Validity in Assessment

PVTs/SVTs, test-taking effort, examiner fidelity

- American Psychological Association, APA Task Force on Psychological Assessment and Evaluation Guidelines. (2020). *APA Guidelines for Psychological Assessment and Evaluation*. Retrieved from <https://www.apa.org/about/policy/guidelines-psychological-assessment-evaluation.pdf>
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Week Beginning April 12: Dissemination/Implementation

Conceptual frameworks, measurement challenges, reporting standards

- *Proctor, E., Silmere, H., Raghavan, R., Hovmand, P., Aarons, G., Bunger, A., Griffey, R., & Hensley, M. (2011). Outcomes for implementation research: Conceptual distinctions and measurement challenges. *Administration and Policy in Mental Health and Mental Health Services Research*, 38(2), 65–76. <https://doi.org/10.1007/s10488-010-0319-7>
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Week Beginning April 19: Multicultural & Ethical Considerations in Assessment

Race-based norms, bias critiques, cultural validity in tests

- *American Psychological Association. (2017). Multicultural guidelines: An ecological approach to context, identity, and intersectionality. *American Psychologist*, 72(9), 823–834. <https://doi.org/10.1037/amp0000121>
- *Helms, J. E., Jernigan, M., & Mascher, J. (2005). The meaning of race in psychology and how to change it. *American Psychologist*, 60(1), 27–36. <https://doi.org/10.1037/0003-066X.60.1.27>
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- International Test Commission. (2018). The ITC guidelines for translating and adapting tests (Second edition). *International Journal of Testing*, 18(2), 101–134. <https://doi.org/10.1080/15305058.2017.1398166>
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Week Beginning April 26: Integrative Clinical Assessment

From measurement to mechanisms to implementation

- *Borsboom, D. (2017). A network theory of mental disorders. *Annual Review of Clinical Psychology*, 13, 91–121. <https://doi.org/10.1146/annurev-clinpsy-032816-045032>
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- *Fisher, A. J., Medaglia, J. D., & Jeronimus, B. F. (2018). Lack of group-to-individual generalizability is a threat to human subjects research. *Proceedings of the National Academy of Sciences*, 115(27), E6106–E6115. <https://doi.org/10.1073/pnas.1711978115>

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Week Beginning May 3: Translational Synthesis

Capstone

- *Cuthbert, B. N., & Insel, T. R. (2013). Toward the future of psychiatric diagnosis: The RDoC framework. *BMC Medicine*, 11, 126. <https://doi.org/10.1186/1741-7015-11-126>
- *Onken, L. S., Carroll, K. M., Shoham, V., Cuthbert, B. N., & Riddle, M. (2014). Reenvisioning clinical science: Unifying the discipline to improve the public health. *Clinical Psychological Science*, 2(1), 22–34. <https://doi.org/10.1177/2167702613497932>
- *Hayes, S. C., & Hofmann, S. G. (2021). “Third-wave” cognitive and behavioral therapies and the emergence of a process-based approach to intervention in psychiatry. *World Psychiatry*, 20(3), 363–375. <https://doi.org/10.1002/wps.20884>
- Collins, L. M., Kugler, K. C., & Gwadz, M. V. (2016). Optimization of multicomponent behavioral interventions (MOST framework). *Translational Behavioral Medicine*, 6(1), 37–46. <https://doi.org/10.1007/s13142-016-0392-8>
- Lyon, A. R., & Koerner, K. (2016). User-centered design for psychosocial intervention development and implementation. *Clinical Psychology: Science and Practice*, 23(2), 180–200. <https://doi.org/10.1111/cpsp.12154>
- Kraemer, H. C. (2016). Messages for clinical scientists in a changing environment. *JAMA Psychiatry*, 73(10), 983–984. <https://doi.org/10.1001/jamapsychiatry.2016.2172>

Assignments and Exams

Psychometrics Project: This project will deepen your understanding of psychometric principles by applying them to an existing psychological measure. By evaluating reliability, validity, and cultural considerations, you will learn to critically assess tools that are used in both research and clinical settings. Choose one widely used psychological instrument (e.g., MMPI-2, PAI, WAIS, CBCL, SCID). Using available datasets (or simulated data sets), compute and interpret reliability estimates (e.g., Cronbach’s alpha, omega, test-retest) and evaluate aspects of validity (construct, convergent, discriminant) using correlation or factor-analytic approaches. Based on your findings, write a report of 8-10 pages in APA style. The report should provide a critical evaluation of the measure’s strengths and limitations, including cross-cultural considerations and potential sources of bias, and include recommendations for appropriate use of the measure in clinical research. Include statistical output in an appendix (tables/figures not counted toward page limit). Projects are due May 3rd and should be submitted as a PDF. Projects will be evaluated on a 100-point scale as follows:

Accuracy of analyses and interpretation (30 points)
Integration of psychometric theory with results (25 points)
Critical evaluation of measure’s strengths/limitations (20 points)

Writing clarity, organization, and APA adherence (15 points)
Quality of tables/figures and code documentation (10 points)

Asynchronous, Self-paced Labs: Each lab requires demonstration of specific competencies (e.g., microskills in rapport building) that are grounded in empirical research and best practices. Labs may be repeated as many times as necessary until the required competency threshold is met. Successful completion is determined by demonstrated mastery rather than time spent, reflecting the course's emphasis on evidence-based training, mastery learning, and skill acquisition consistent with a clinical science model. The following four labs must be accomplished (recommend budgeting 3-4 weeks per lab). Each lab is graded pass/fail. The lab requirement counts for 20% of the final course grade. Students earn these points only if all four labs are passed. Passing fewer than four labs results in 0% awarded for the lab requirement.

Core Clinical Skills Lab I

Microskills with simulated patients

Minimal encouragements (e.g., brief verbal or nonverbal cues to continue), asking questions (both open and closed), paraphrasing (restating client content), reflection of feeling (mirroring emotional content), concreteness (focusing on specific details), summarizing (integrating and recapping session material), and situation clarification (helping the client clarify circumstances or experiences)

- Ajluni V. (2025). Artificial intelligence in psychiatric education: Enhancing clinical competence through simulation. *Industrial Psychiatry Journal*, 34(1), 11–15. https://doi.org/10.4103/ipj.ipj_377_24
- Schaffrath, J., Hehlmann, M. I., Mink, F., Moggia, D., & Lutz, W. (2025). Using latent profile analysis of clinical microskills to predict alliance and outcome of cognitive behavioral therapy. *Psychotherapy Research: Journal of the Society for Psychotherapy Research*, 1–16. <https://doi.org/10.1080/10503307.2025.2528029>
- Lee, E., Greenblatt, A., Hu, R., Johnstone, M., & Kourgiantakis, T. (2022). Microskills of broaching and bridging in cross-cultural psychotherapy: Locating therapy skills in the epistemic domain toward fostering epistemic justice. *The American Journal of Orthopsychiatry*, 92(3), 310–321. <https://doi.org/10.1037/ort0000610>

Core Clinical Skills Lab II

More microskills with simulated patients

Accurate empathy, confrontation (gently challenging discrepancies or defenses), positive relabeling (reframing client experiences in a more adaptive light), therapist self-disclosure (judiciously sharing relevant personal information), and directness (clear, unambiguous communication).

- Stolorowicz-Melman, D., Lazarus, G., & Atzil-Slonim, D. (2023). Is empathic accuracy enough? The role of therapists' interventions in the associations between empathic accuracy and session outcome. *Journal of Counseling Psychology*, 70(6), 682–690. <https://doi.org/10.1037/cou0000708>
- Yamin, J. B., Cannoy, C. N., Gibbins, K. M., Ferenczi, N., Moralez, R., & Greenberg, L. S. (2023). Experiential training of mental health graduate students in emotional processing

skills: A randomized, controlled trial. *Psychotherapy*, 60(4), 512–524.
<https://doi.org/10.1037/pst0000495>

- Köhler, S., Guhn, A., Betzler, F., Schnell, K., Georgi, A., & Köhler, T. (2017). Therapeutic self-disclosure within DBT, schema therapy, and CBASP: Opportunities and challenges. *Frontiers in Psychology*, 8, 2073. <https://doi.org/10.3389/fpsyg.2017.02073>
- Pinto-Coelho, K. G., Hill, C. E., Kearney, M. S., Kivlighan, D. M., Gelso, C. J., & Silberschatz, G. (2018). When in doubt, sit quietly: A qualitative investigation of experienced therapists' perceptions of self-disclosure. *Journal of Counseling Psychology*, 65(4), 440–452. <https://doi.org/10.1037/cou0000288>
- Hatcher, R. L. (2015). Interpersonal competencies: Responsiveness, technique, and training in psychotherapy. *American Psychologist*, 70(8), 747–757.
<https://doi.org/10.1037/a0039803>

Core Clinical Skills Lab III

Cross-cultural microskills with simulated patients

Broaching (initiating discussion of cultural differences), bridging (connecting cultural experiences to therapeutic work), and cultural immediacy (responding in the moment to cultural dynamics and cues that emerge during the session)

- Lee, E., Greenblatt, A., Hu, R., Johnstone, M., & Kourgiantakis, T. (2022). Microskills of broaching and bridging in cross-cultural psychotherapy: Locating therapy skills in the epistemic domain toward fostering epistemic justice. *The American Journal of Orthopsychiatry*, 92(3), 310–321. <https://doi.org/10.1037/ort0000610>

Core Clinical Skills Lab IV

Intakes with simulated patients

Phone triage, screening, referring out, informed consent, risk assessment, confidentiality, and behavioral observations

- Saidinejad, M., Duffy, S., Wallin, D., Vukovic, A. A., Mishra, S., & Grupp-Phelan, J. (2023). The management of children and youth with pediatric mental and behavioral health emergencies. *Pediatrics*, 152(3), e2023063256. <https://doi.org/10.1542/peds.2023-063256>
- Gryglewicz, K., Monahan, M. M., Chen, J. I., Karver, M. S., Knapp, S., & Wirshing, D. A. (2020). Examining the effects of role play practice in enhancing clinical skills to assess and manage suicide risk. *Journal of Mental Health*, 29(5), 549–557.
<https://doi.org/10.1080/09638237.2018.1521936>

Final Exam: The course concludes with a comprehensive take-home final examination. This exam is designed to assess each student's ability to integrate concepts from across the semester. The exam emphasizes integration of key themes covered in class: philosophy of science and evidence hierarchies, construct validity and psychometrics, measurement-based care, research-practice integration, mediators and moderators of change, structured and semi-structured interviewing, intelligence and achievement testing, personality and self-report measures, assessment fidelity, multicultural and ethical considerations, and introductory dissemination and implementation frameworks. The exam will consist of essay-style questions and applied analysis prompts that require students to (a) evaluate the psychometric and

cultural adequacy of assessment approaches, (b) connect assessment to mechanism science and outcome monitoring, and (c) consider translational pathways from measurement to implementation. Students are expected to demonstrate critical thinking, integration of empirical evidence, and scholarly writing consistent with doctoral-level training. The final exam is cumulative, closed to collaboration, and must be completed within the assigned time frame. Exams will be made available April 30th and are due May 10th.

Grading

Final grades will be determined by weighting the earned scores on the assignments and exams as follows:

Requirement	Weight in Final Grade	Letter	Percentage	Letter	Percentage
Class Participation	15%	A	93-100	C	73-76
Asynchronous Labs (Pass/Fail)	20%	A-	90-92	C-	70-72
Psychometrics Project	35%	B+	87-89	D+	67-69
Final Exam	50%	B	83-86	D	63-66
		B-	80-82	D-	60-62
		C+	77-79	F	59 or lower

All assigned grades will adhere to the allocations above and will not be changed upon request. Any grade dispute must be raised within one week of the posting of the grade in question. Grades will be posted on eLearning.

Course Policies

Incompletes: Incompletes will only be given if the only remaining requirement is completion of the final exam. An incomplete grade would need to be arranged prior to the final exam and would require approval of the instructor based upon a substantial excuse.

Extra Credit: There is no extra credit in this course.

Late Work: A penalty of one letter grade will be assessed for each calendar day late.

Communication

This course utilizes online tools for interaction and communication. Some external communication tools such as regular email and a web conferencing tool may also be used during the semester. For more details, please visit the [Student eLearning Tutorials](#) webpage for video demonstrations on eLearning tools. Please use email or office hours to ask questions of the instructor. Student emails will be answered within 3 working days under normal circumstances.

Class Materials

The instructor may provide class materials that will be made available to all students registered for this class as they are intended to supplement the classroom experience. These materials

may be downloaded during the course; however, these materials are for registered students' use only. Class materials may not be reproduced or shared with those not in class or uploaded to other online environments except to implement an approved AccessAbility Resource Center accommodation. Failure to comply with these University requirements is a violation of the [Student Code of Conduct](#).

Class Attendance

Regular and punctual class attendance is expected. Students who fail to attend class regularly are inviting scholastic difficulty.

Class Participation

Regular class participation is expected. Students who fail to participate in class regularly are inviting scholastic difficulty. A portion of the grade for this course is directly tied to your participation in this class. It also includes engaging in group or other activities during class that solicit your feedback on homework assignments, readings, or materials covered in the lectures (and/or labs). Class participation is documented by faculty. Successful participation is defined as consistently adhering to University requirements, as presented in this syllabus. Failure to comply with these University requirements is a violation of the [Student Code of Conduct](#).

Texas Senate Bill 17, the recent law that prohibits diversity, equity, and inclusion programs and activities at public universities in Texas, does not in any way apply to academic course instruction. Students should not feel the need to self-censor or limit their participation in academic courses pertaining to topics of race and racism, structural inequality, LGBTQ+ issues, or diversity, equity, and inclusion, and related topics.

Class Recordings

Students are expected to follow appropriate University policies and maintain the security of passwords used to access recorded lectures. Unless the AccessAbility Resource Center has approved the student to record the instruction, students are expressly prohibited from recording any part of this course. Recordings may not be published, reproduced, or shared with those not in the class, or uploaded to other online environments except to implement an approved AccessAbility Resource Center accommodation. Failure to comply with these University requirements is a violation of the [Student Code of Conduct](#).

The instructor may record meetings of this course. These recordings will be made available to all students registered for this class if the intent is to supplement the classroom experience. If the instructor or a UTD school/department/office plans any other uses for the recordings, consent of the students identifiable in the recordings is required prior to such use unless an exception is allowed by law.

Academic Integrity / Misconduct

Students are reminded that academic misconduct will not be tolerated. For example: (a) copying another student's work, in part or in whole; (b) unauthorized communication to obtain information about a test(s); (c) using unauthorized material during a test; (d) handing in any

work product that is not your own; (e) signing someone else's name; (f) failing to give credit to the intellectual property of another in any written work (including, but not limited to, plagiarizing). For a more extensive listing of behaviors that are considered misconduct please refer to the UT Dallas [Community Standards and Conduct](#).

Comet Creed

This creed was voted on by the UT Dallas student body in 2014. It is a standard that Comets choose to live by and encourage others to do the same:

"As a Comet, I pledge honesty, integrity, and service in all that I do."

Accommodations for Students with Disabilities

Please review [the section](#) within the UT Dallas Syllabus Policies and Procedures webpage.

Server Unavailability or Other Technical Difficulties

The University is committed to providing a reliable learning management system to all users. However, in the event of any unexpected server outage or any unusual technical difficulty which prevents students from completing a time sensitive assessment activity, the instructor will provide an appropriate accommodation based on the situation. Students should immediately report any problems to the instructor and contact the online [eLearning Help Desk](#). The instructor and the eLearning Help Desk will work with the student to resolve any issues at the earliest possible time.

Academic Support Resources

Please visit the [Academic Support Resources](#) page to view the University's academic support resources for all students.

UT Dallas Syllabus Policies and Procedures

The information contained in the following link constitutes the University's policies and procedures segment of the course syllabus. Please review the catalog sections regarding the [credit/no credit](#) or [pass/fail](#) grading option and withdrawal from class. Please go to [UT Dallas Syllabus Policies](#) webpage for these policies.

The descriptions and timelines contained in this syllabus are subject to change at the discretion of the Professor.

CPSY 7310: Clinical Psychological Science II
Clinical Science of Intervention: Outcomes, ESTs, and Implementation

Course Pre-requisites, Co-requisites, and/or Other Restrictions

CPSY 7300. Department consent required.

Course Description

This course provides advanced training in the clinical science of intervention, emphasizing the design, evaluation, and dissemination of evidence-based treatments. Students engage deeply with primary research on treatment-outcome designs (including RCTs, dismantling and adaptive trials, and meta-analytic synthesis), empirically supported treatments (ESTs), transdiagnostic and modular approaches, and process-based CBT. Special emphasis is placed on mechanisms of change and active ingredients of therapy, with students critically evaluating mediator and moderator research and applying quantitative methods to intervention data. The course also addresses advanced dissemination and implementation science, supervision as an evidence-based practice, systems of care, and cultural/contextual adaptations of interventions. Major assignments include an NIH-style Grant Proposal, requiring students to design and defend a novel intervention study, and a cumulative final exam, ensuring that students demonstrate competence in designing, critiquing, and disseminating intervention science at the level expected of clinical psychological scientists.

Student Learning Objectives/Outcomes

Upon completion of this course, Students will be able to:

- Critically appraise intervention literatures, including treatment-outcome studies, meta-analyses, and comparative effectiveness trials.
- Design rigorous intervention studies that test mechanisms, mediators, moderators, and dissemination strategies.
- Evaluate methodological rigor in intervention science, including adequacy of trial designs, control conditions, fidelity/adaptation balance, and outcome measurement.
- Apply quantitative methods (e.g., mediation, moderation, longitudinal modeling) to analyze intervention mechanisms and processes of change.
- Develop and defend an NIH-style Grant Proposal that advances intervention science through the design of an innovative study.
- Integrate cultural, developmental, and contextual considerations into intervention design, adaptation, and dissemination to ensure equity and scientific relevance.

Successful completion of this course is expected to expand competency in the following areas:

- Scientific Mindedness
- Scientific Foundations
- Knowledge of Ethical, Legal and Professional Standards and Guidelines
- Awareness and Application of Ethical Decision Making
- Empowerment
- Diagnosis
- Measurement and Psychometrics

- Application of Assessment Methods
- Diagnostic Conceptualization and Recommendations
- Intervention Planning
- Intervention Implementation
- Knowledge of Interventions
- Scientific Foundation of Professional Practice
- Treatment Monitoring
- Professional Identity
- Accountability
- Interpersonal Relationships
- Expressive Skills
- Affective Skills
- Supervision Skills Development
- Understands How Participation in Interdisciplinary Collaboration/ Consultation Enhances Outcomes

Course Access and Navigation

This course can be accessed using your UT Dallas NetID account on the [eLearning](#) website. Please see the course access and navigation section of the [Getting Started with eLearning](#) webpage for more information. To become familiar with the eLearning tool, please see the [Student eLearning Tutorials](#) webpage. UT Dallas provides eLearning technical support 24 hours a day, 7 days a week. The [eLearning Support Center](#) includes a toll-free telephone number for immediate assistance (1-866-588-3192), email request service, and an online chat service.

Course Calendar & Readings

The weekly reading lists in CPSY 7310 are intentionally expansive to reflect the breadth of salient scholarship. Each week, the instructor will designate a subset of core readings as required anchors for discussion, while the broader list functions as a curated library that students are expected to draw from when preparing their psychometrics project and final exam. All readings are provided on the eLearning course site.

Fall 2027 Weekly Schedule

Week Beginning August 23: Intervention Science Foundations

History, philosophy, research-to-practice gap

- *Eysenck, H. J. (1952). The effects of psychotherapy: An evaluation. *Journal of Consulting Psychology*, 16(5), 319–324.
- *Smith, M. L., & Glass, G. V. (1977). Meta-analysis of psychotherapy outcome studies. *American Psychologist*, 32(9), 752–760. <https://doi.org/10.1037/0003-066X.32.9.752>
- Wampold, B. E., Mondin, G. W., Moody, M., Stich, F., Benson, K., & Ahn, H.-N. (1997). A meta-analysis of outcome studies comparing bona fide psychotherapies: Empirically, “all must have prizes.” *Psychological Bulletin*, 122(3), 203–215. <https://doi.org/10.1037/0033-2909.122.3.203>
- *Kazdin, A. E. (2008). Evidence-based treatment and practice: New opportunities to bridge clinical research and practice. *American Psychologist*, 63(3), 146–159. <https://doi.org/10.1037/0003-066X.63.3.146>

- McHugh, R. K., & Barlow, D. H. (2010). The dissemination and implementation of evidence-based psychological treatments: A review of current efforts. *American Psychologist*, 65(2), 73–84. <https://doi.org/10.1037/a0018121>
- Garland, A. F., Brookman-Frazee, L., Hurlburt, M. S., Accurso, E. C., Zoffness, R., Haine-Schlagel, R., & Ganger, W. (2010). Mental health care for children with disruptive behavior problems: A view inside therapists' offices. *Psychiatric Services*, 61(8), 788–795. <https://doi.org/10.1176/ps.2010.61.8.788>

Week Beginning August 30: Treatment-Outcome Research Methods (Part 1)

RCTs, control conditions, dismantling/adaptive designs

- Schulz, K. F., Altman, D. G., & Moher, D. (2010). CONSORT 2010 statement: Updated guidelines for reporting parallel group randomized trials. *BMJ*, 340, c332. <https://doi.org/10.1136/bmj.c332>
- *Mohr, D. C., Spring, B., Freedland, K. E., Beckner, V., Arean, P., Hollon, S. D., ... Kaplan, R. (2009). The selection and design of control conditions for randomized controlled trials of psychological interventions. *Psychosomatic Medicine*, 71(2), 197–204. <https://doi.org/10.1097/PSY.0b013e318198cd76>
- *Bhatt, D. L., & Mehta, C. (2016). Adaptive designs for clinical trials. *The New England Journal of Medicine*, 375(1), 65–74. <https://doi.org/10.1056/NEJMr1510061>
- *Pallmann, P., Bedding, A. W., Choodari-Oskoei, B., Dimairo, M., Flight, L., Hampson, L. V., Holmes, J., Mander, A. P., Sydes, M. R., Villar, S. S., Wason, J., Weir, C. J., Yap, C., & Jaki, T. (2018). Adaptive designs in clinical trials: Why use them, and how to run and report them. *BMC Medicine*, 16(1), 29. <https://doi.org/10.1186/s12916-018-1017-7>
- Jacobson, N. S., Dobson, K. S., Truax, P. A., Addis, M. E., Koerner, K., Gollan, J. K., ... Prince, S. E. (1996). A component analysis of cognitive-behavioral treatment for depression. *Journal of Consulting and Clinical Psychology*, 64(2), 295–304. <https://doi.org/10.1037//0022-006x.64.2.295>
- Murphy, S. A. (2005). An experimental design for the development of adaptive treatment strategies. *Statistics in Medicine*, 24(10), 1455–1481. <https://doi.org/10.1002/sim.2022>
- Almirall, D., Nahum-Shani, I., Sherwood, N. E., & Murphy, S. A. (2014). Introduction to SMART designs for developing and testing adaptive interventions. *Translational Behavioral Medicine*, 4(3), 260–274. <https://doi.org/10.1007/s13142-014-0265-0>
- Collins, L. M., Murphy, S. A., & Strecher, V. (2007). The Multiphase Optimization Strategy (MOST) and SMART: New methods for more potent eHealth interventions. *American Journal of Preventive Medicine*, 32(5 Suppl), S112–S118. <https://doi.org/10.1016/j.amepre.2007.01.022>

Week Beginning September 6: Treatment-Outcome Research Methods (Part 2)

Trial validity, outcome metrics, systematic reviews/meta-analyses

- Jacobson, N. S., & Truax, P. (1991). Clinical significance: A statistical approach to defining meaningful change in psychotherapy research. *Journal of Consulting and Clinical Psychology*, 59(1), 12–19. <https://doi.org/10.1037/0022-006X.59.1.12>
- Kazdin, A. E. (1999). The meanings and measurement of clinical significance. *Journal of Consulting and Clinical Psychology*, 67(3), 332–339. <https://doi.org/10.1037/0022-006X.67.3.332>

- Turner, E. H., Matthews, A. M., Linardatos, E., Tell, R. A., & Rosenthal, R. (2008). Selective publication of antidepressant trials and its influence on apparent efficacy. *New England Journal of Medicine*, 358(3), 252–260. <https://doi.org/10.1056/NEJMsa065779>
- Higgins, J. P. T., & Thompson, S. G. (2002). Quantifying heterogeneity in a meta-analysis. *Statistics in Medicine*, 21(11), 1539–1558. <https://doi.org/10.1002/sim.1186>
- Lakens, D. (2017). Equivalence tests: A practical primer for t tests, correlations, and meta-analyses. *Social Psychological and Personality Science*, 8(4), 355–362. <https://doi.org/10.1177/1948550617697177>
- *Leichsenring, F., Steinert, C., Rabung, S., & Ioannidis, J. P. A. (2022). The efficacy of psychotherapies and pharmacotherapies for mental disorders in adults: An umbrella review and meta-analytic evaluation of recent meta-analyses. *World Psychiatry*, 21(1), 133–145. <https://doi.org/10.1002/wps.20941>
- *Harrer, M., Cuijpers, P., Schuurmans, L. K. J., & others. (2023). Evaluation of randomized controlled trials: A primer and tutorial for mental health researchers. *Trials*, 24(1), 562. <https://doi.org/10.1186/s13063-023-07596-3>
- *Cuijpers, P., Miguel, C., Ciharova, M., & others. (2024). Absolute and relative outcomes of psychotherapies for eight mental disorders: A systematic review and meta-analysis. *World Psychiatry*, 23(2), 267–275. <https://doi.org/10.1002/wps.21203>
- Kraemer, H. C., & Kupfer, D. J. (2006). Size of treatment effects and their clinical significance. *Biological Psychiatry*, 59(11), 990–996. <https://doi.org/10.1016/j.biopsych.2005.09.014>

Week Beginning September 13: Psychotherapy Models & Orientations as Hypothesis Generators

From brands to processes — the mechanistic legacy of the “schools”

- *Shedler, J. (2010). The efficacy of psychodynamic psychotherapy. *American Psychologist*, 65(2), 98–109. <https://doi.org/10.1037/a0018378>
- Crits-Christoph, P. (1992). The efficacy of brief dynamic psychotherapy: A meta-analysis. *American Journal of Psychiatry*, 149(2), 151–158. <https://doi.org/10.1176/ajp.149.2.151>
- *Hofmann, S. G., Asnaani, A., Vonk, I. J. J., Sawyer, A. T., & Fang, A. (2012). The efficacy of cognitive behavioral therapy: A review of meta-analyses. *Cognitive Therapy and Research*, 36(5), 427–440. <https://doi.org/10.1007/s10608-012-9476-1>
- David, D., Cristea, I. A., & Hofmann, S. G. (2018). Why cognitive behavioral therapy is the current gold standard of psychotherapy. *Frontiers in Psychiatry*, 9, 4. <https://doi.org/10.3389/fpsy.2018.00004>
- Cuijpers, P., Reijnders, M., & Huibers, M. J. H. (2019). The Role of Common Factors in Psychotherapy Outcomes. *Annual review of clinical psychology*, 15, 207–231. <https://doi.org/10.1146/annurev-clinpsy-050718-095424>
- *Goldfried, M. R. (2019). Obtaining consensus in psychotherapy: What holds us back? *American Psychologist*, 74(4), 484–496. <https://doi.org/10.1037/amp0000474>

Week Beginning September 20: Mechanisms & Active Ingredients of Therapy

Conceptual, methodological, and empirical process-outcome links

- *Kazdin, A. E. (2009). Understanding how and why psychotherapy leads to change: Mediators, mechanisms, and causal pathways. *Annual Review of Psychology*, 60, 1–27. <https://doi.org/10.1146/annurev.psych.60.110707.163451>

- Tang, T. Z., & DeRubeis, R. J. (1999). Sudden gains and cognitive therapy for depression. *Journal of Consulting and Clinical Psychology*, 67(6), 894–904. <https://doi.org/10.1037/0022-006X.67.6.894>
- *Zilcha-Mano, S. (2017). Is the alliance really therapeutic? Revisiting the question. *American Psychologist*, 72(4), 311–325. <https://doi.org/10.1037/a0040435>
- *Webb, C. A., DeRubeis, R. J., & Barber, J. P. (2010). Therapist adherence/competence and treatment outcome: A meta-analytic review. *Journal of Consulting and Clinical Psychology*, 78(2), 200–211. <https://doi.org/10.1037/a0018912>
- Hofmann, S. G., Sawyer, A. T., Fang, A., & Asnaani, A. (2012). Emotion dysregulation model of mood and anxiety disorders. *Depression and Anxiety*, 29(5), 409–416. <https://doi.org/10.1002/da.21888>
- Lemmens, L. H. J. M., Müller, V. N. L. S., Arntz, A., & Huibers, M. J. H. (2016). Mechanisms of change in cognitive therapy and interpersonal psychotherapy for depression: A randomized dismantling study. *Journal of Consulting and Clinical Psychology*, 84(7), 623–635. <https://doi.org/10.1037/ccp0000129>

Week Beginning September 27: Empirically Supported Treatments (ESTs, Part 1)

APA guidelines, comparative effectiveness, EST history

- Chambless, D. L., & Hollon, S. D. (1998). Defining empirically supported therapies. *Journal of Consulting and Clinical Psychology*, 66(1), 7–18. <https://doi.org/10.1037/0022-006X.66.1.7>
- *Chambless, D. L., & Ollendick, T. H. (2001). Empirically supported psychological interventions: Controversies and evidence. *Annual Review of Psychology*, 52, 685–716. <https://doi.org/10.1146/annurev.psych.52.1.685>
- American Psychological Association Presidential Task Force on Evidence-Based Practice. (2006). Evidence-based practice in psychology. *American Psychologist*, 61(4), 271–285. <https://doi.org/10.1037/0003-066X.61.4.271>
- Tolin, D. F. (2010). Is cognitive-behavioral therapy more effective than other therapies? A meta-analytic review. *Clinical Psychology Review*, 30(6), 710–720. <https://doi.org/10.1016/j.cpr.2010.05.003>
- Wampold, B. E., & Budge, S. L. (2012). The 2011 Leona Tyler Award address: The therapeutic relationship and evidence-based practice. *The Counseling Psychologist*, 40(5), 601–623. <https://doi.org/10.1177/0011000011432102>
- *Cuijpers, P., Karyotaki, E., de Wit, L., & Ebert, D. D. (2020). The effects of fifteen evidence-supported therapies for adult depression: A meta-analytic review. *Psychotherapy Research*, 30(3), 279–293. <https://doi.org/10.1080/10503307.2019.1649732>

Week Beginning October 4: ESTs (Part 2)

Transdiagnostic and modular treatments, tech-supported ESTs

- *Barlow, D. H., Farchione, T. J., Bullis, J. R., Gallagher, M. W., Murray-Latin, H., Sauer-Zavala, S., Bentley, K. H., Thompson-Hollands, J., Conklin, L. R., Boswell, J. F., Ametaj, A., Carl, J. R., Boettcher, H. T., & Cassiello-Robbins, C. (2017). The Unified Protocol for transdiagnostic treatment of emotional disorders compared with diagnosis-specific protocols for anxiety disorders: A randomized clinical trial. *JAMA Psychiatry*, 74(9), 875–884. <https://doi.org/10.1001/jamapsychiatry.2017.2164>

- *Jeppesen, P., Wolf, R. T., Nielsen, S. M., Børgesen, M. N., Bilenberg, N., Christensen, T. Ø., Eriksen, T., Thomsen, P. H., Simonsen, E., & Arnfred, J. (2021). Effectiveness of transdiagnostic cognitive-behavioral psychotherapy compared with management as usual for youth with common mental health problems: A randomized clinical trial. *JAMA Psychiatry*, 78(3), 250–260. <https://doi.org/10.1001/jamapsychiatry.2020.4045>
- *Newby, J. M., Twomey, C., Yuan Li, S. S., & Andrews, G. (2016). Transdiagnostic computerised cognitive behavioural therapy for depression and anxiety: A systematic review and meta-analysis. *Journal of Affective Disorders*, 199, 30–41. <https://doi.org/10.1016/j.jad.2016.03.018>
- McEvoy, P. M., Nathan, P., & Norton, P. J. (2009). Efficacy of transdiagnostic treatments: A review of published outcome studies and future research directions. *Clinical Psychology Review*, 29(2), 173–182. <https://doi.org/10.1016/j.cpr.2008.12.005>
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- Andersson, G., Cuijpers, P., Carlbring, P., Riper, H., & Hedman, E. (2014). Guided Internet-based cognitive behavioral therapy for psychiatric disorders: A systematic review and meta-analysis. *Cognitive Behaviour Therapy*, 43(3), 223–232. <https://doi.org/10.1080/16506073.2014.916396>
- Firth, J., Torous, J., Nicholas, J., Carney, R., Rosenbaum, S., & Sarris, J. (2017). The efficacy of smartphone-based mental health interventions for depressive symptoms: A meta-analysis of randomized controlled trials. *World Psychiatry*, 16(3), 287–298. <https://doi.org/10.1002/wps.20472>
- Parsons, T. D. (2022). Ethics in technology for clinical psychology. In G. Asmundson (Ed.) *Comprehensive Clinical Psychology* (2nd ed., Vol. 10, pp. 307–320). Elsevier. <https://dx.doi.org/10.1016/B978-0-12-818697-8.00100-X>
- Karyotaki, E., Kemmeren, L., Riper, H., Twisk, J., Hoogendoorn, A., Kleiboer, A., ... Cuijpers, P. (2021). Is self-guided Internet-based cognitive behavioral therapy as effective as guided Internet-based cognitive behavioral therapy for depression? A meta-analysis. *JAMA Psychiatry*, 78(4), 361–371. <https://doi.org/10.1001/jamapsychiatry.2020.4364>

Week Beginning October 11: Process-Based CBT (Part 1)

Theory, conceptual models, functional analysis

- *Hayes, S. C., Hofmann, S. G., & Ciarrochi, J. (2020). A process-based approach to psychological diagnosis and treatment: Implications for assessment, case formulation, and intervention. *Behavior Therapy*, 51(6), 987–1006. <https://doi.org/10.1016/j.beth.2020.06.006>
- Hofmann, S. G., Curtiss, J., Hayes, S. C., & Hayes, C. (2021). Beyond packaged protocols: Process-based therapy. *Cognitive Therapy and Research*, 45(2), 265–276. <https://doi.org/10.1007/s10608-020-10166-4>
- Mansell, W., Harvey, A., Watkins, E. R., & Shafran, R. (2009). Conceptual foundations of the transdiagnostic approach to CBT. *Journal of Cognitive Psychotherapy*, 23(1), 6–19. <https://doi.org/10.1891/0889-8391.23.1.6>

- *Hofmann, S. G., & Hayes, S. C. (2018). The future of intervention science: Process-based rather than protocol-based care. *Clinical Psychological Science*, 6(3), 395–404. <https://doi.org/10.1177/2167702618772296>
- Levin, M. E., Krafft, J., Hicks, E. T., Pierce, B. G., & Twohig, M. P. (2021). Psychological flexibility as a transdiagnostic process of change in CBT: A meta-analysis. *Behavior Therapy*, 52(3), 612–629. <https://doi.org/10.1016/j.beth.2020.09.005>
- *Hayes, S. C., Hofmann, S. G., Stanton, C. E., Carpenter, J. K., Sanford, B. T., Curtiss, J. E., & Ciarrochi, J. (2019). The role of the individual in the coming era of process-based therapy. *Behaviour Research and Therapy*, 117, 40–53. <https://doi.org/10.1016/j.brat.2018.10.005>

Week Beginning October 18: Process-Based CBT (Part 2)

Techniques, personalization, engagement

- *Fisher, A. J., & Boswell, J. F. (2016). Enhancing the personalization of CBT: From data to practice. *Behavior Therapy*, 47(6), 835–849. <https://doi.org/10.1016/j.beth.2016.06.004>
- *Lutz, W., Hofmann, S. G., Rubel, J., Boswell, J. F., Shear, M. K., Gorman, J. M., & Barlow, D. H. (2014). Patterns of early change and treatment outcome in CBT for panic disorder. *Journal of Consulting and Clinical Psychology*, 82(2), 287–299. <https://doi.org/10.1037/a0035535>
- *Delgadillo, J., & Groom, M. (2017). Using machine learning to predict therapy outcomes and guide personalized care in mental health services. *Behavior Research and Therapy*, 99, 157–165. <https://doi.org/10.1016/j.brat.2017.10.003>
- Zilcha-Mano, S., Zhu, X., & Barber, J. P. (2018). Personalized treatment in psychotherapy: The importance of treatment fits. *Clinical Psychology: Science and Practice*, 25(4), e12247. <https://doi.org/10.1111/cpsp.12247>
- Lorenzo-Luaces, L., DeRubeis, R. J., & Webb, C. A. (2014). A foreseeable future for personalized care in depression. *Clinical Psychology: Science and Practice*, 21(4), 339–351. <https://doi.org/10.1111/cpsp.12091>
- Hayes, S. C., & Hofmann, S. G. (2021). “Third-wave” cognitive and behavioral therapies and the emergence of a process-based approach to intervention in psychiatry. *World Psychiatry*, 20(3), 363–375. <https://doi.org/10.1002/wps.20884>
- Wade, T. D., & Waller, G. (2025). Ten generic competences to improve outcomes of cognitive behaviour therapy: Evidence, postulated processes, and clinical implications. *Behaviour Research and Therapy*, 193, 104826. <https://doi.org/10.1016/j.brat.2025.104826>
- Kazantzis, N. (2024). Cognitive behavioral therapy: Strategies for enhancing treatment engagement. *The Psychiatric Clinics of North America*, 47(2), 301–310. <https://doi.org/10.1016/j.psc.2024.03.002>
- Walter, H. J., Bukstein, O. G., Abright, A. R., Keable, H., Ramtekkar, U., Ripperger-Suhler, J., Rockhill, C., & Wolraich, M. (2020). Clinical practice guideline for the assessment and treatment of children and adolescents with anxiety disorders. *Journal of the American Academy of Child & Adolescent Psychiatry*, 59(10), 1107–1124. <https://doi.org/10.1016/j.jaac.2020.05.005>

Week Beginning October 25: Dissemination/Implementation (Revisited Anew)

Hybrid effectiveness-implementation designs, implementation strategies (ERIC), the Consolidated Framework for Implementation Research (CFIR), sustainability, scaling, and equity in health and public mental health interventions

- *Curran, G. M., Bauer, M., Mittman, B., Pyne, J. M., & Stetler, C. (2012). Effectiveness-implementation hybrid designs: Combining elements of clinical effectiveness and implementation research to enhance public health impact. *Medical Care*, 50(3), 217–226. <https://doi.org/10.1097/MLR.0b013e3182408812>.
- *Powell, B. J., Waltz, T. J., Chinman, M. J., Damschroder, L. J., Smith, J. L., Matthieu, M. M., ... Kirchner, J. E. (2015). A refined compilation of implementation strategies: Results from the ERIC project. *Implementation Science*, 10, 21. <https://doi.org/10.1186/s13012-015-0209-1>.
- Damschroder, L. J., Reardon, C. M., Opra Widerquist, M. A., & Lowery, J. (2022). Conceptualizing outcomes for use with the Consolidated Framework for Implementation Research (CFIR): The CFIR Outcomes Addendum. *Implementation Science*, 17, 7. <https://doi.org/10.1186/s13012-021-01181-5>.
- Shelton, R. C., Cooper, B. R., & Stirman, S. W. (2018). The sustainability of evidence-based interventions and practices in public health and health care. *Annual Review of Public Health*, 39, 55–76. <https://doi.org/10.1146/annurev-publhealth-040617-014731>.
- Beidas, R. S., Boyd, M., Casline, E., et al. (2025). Harnessing implementation science in clinical psychology: Past, present, and future. *Annual Review of Clinical Psychology*, 21(1), 529–555. <https://doi.org/10.1146/annurev-clinpsy-081423-021727>.
- Corôa, R. C., Gogovor, A., Ben Charif, A., et al. (2023). Evidence on scaling in health and social care: An umbrella review. *The Milbank Quarterly*, 101(3), 881–921. <https://doi.org/10.1111/1468-0009.12649>.
- Baumann, A. A., Shelton, R. C., Kumanyika, S., & Haire-Joshu, D. (2023). Advancing healthcare equity through dissemination and implementation science. *Health Services Research*, 58(Suppl. 3), 327–344. <https://doi.org/10.1111/1475-6773.14175>.
- Ashcraft, L. E., Goodrich, D. E., Hero, J., Bauer, M. S., Miller, C. J., & Kilbourne, A. M. (2024). A systematic review of experimentally tested implementation strategies across health and human service settings: Evidence from 2010–2022. *Implementation Science*, 19(1), 43. <https://doi.org/10.1186/s13012-024-01369-5>
- Reininghaus, U., Reinhold, A. S., Priebe, S., et al. (2024). Toward equitable interventions in public mental health: A review. *JAMA Psychiatry*, 81(12), 1270–1275. <https://doi.org/10.1001/jamapsychiatry.2024.3206>.

Week Beginning November 1: Supervision as a Science

Theoretical, process-oriented, and empirical foundations of supervision and competency development

- *Milne, D., & James, I. (2000). A systematic review of effective cognitive-behavioural supervision. *British Journal of Clinical Psychology*, 39(2), 111–127. <https://doi.org/10.1348/014466500163149>
- Rakovshik, S. G., & McManus, F. (2008). Establishing evidence-based training in cognitive behavioral therapy: A review of evaluation studies and methods. *Clinical Psychology Review*, 28(6), 782–799. <https://doi.org/10.1016/j.cpr.2007.12.008>
- Martino, S., Ball, S. A., Nich, C., Frankforter, T. L., & Carroll, K. M. (2009). Community program therapist adherence and competence in motivational enhancement therapy. *Journal of Substance Abuse Treatment*, 36(3), 224–233. <https://doi.org/10.1016/j.jsat.2008.06.004>
- Webb, C. A., DeRubeis, R. J., & Barber, J. P. (2010). Therapist adherence/competence and treatment outcome: A meta-analytic review. *Journal of Consulting and Clinical Psychology*, 78(2), 200–211. <https://doi.org/10.1037/a0018912>

- Beidas, R. S., Edmunds, J. M., Marcus, S. C., & Kendall, P. C. (2012). Training and consultation to increase cognitive behavioral therapy use in community mental health. *Implementation Science*, 7, 17. <https://doi.org/10.1186/1748-5908-7-17>
- *Newman, C. F. (2010). Competency in conducting cognitive-behavioral therapy: Foundational, functional, and supervisory aspects. *Psychotherapy*, 47(1), 12–19. <https://doi.org/10.1037/a0018849>
- Rupert, P. A. & Pakenham, K. I. (2022). Self-care and burnout: A proactive values-based perspective. In G. Asmundson (Ed.) *Comprehensive Clinical Psychology* (2nd ed., Vol. 2, pp. 138-148). Elsevier. <https://dx.doi.org/10.1016/B978-0-12-818697-8.00100-X>
- Cummings, J. A., Ballantyne, E. C., & Scallion, L. M. (2015). Essential processes for cognitive behavioral clinical supervision: Agenda setting, problem-solving, and formative feedback. *Psychotherapy*, 52(2), 158–163. <https://doi.org/10.1037/a0038712>
- *Alfonsson, S., Parling, T., Spännargård, Å., Andersson, G., & Lundgren, T. (2018). The effects of clinical supervision on supervisees and patients in cognitive behavioral therapy: A systematic review. *Cognitive Behaviour Therapy*, 47(3), 206–228. <https://doi.org/10.1080/16506073.2017.1369559>

Week Beginning November 8: Systems of Care & Team Science

Interdisciplinary models, team science, collaborative care, integration of mental health into primary care

- Unützer, J., Katon, W., Callahan, C. M., Williams, J. W., Hunkeler, E., Harpole, L., ... Langston, C. (2002). Collaborative care management of late-life depression in the primary care setting (IMPACT). *JAMA*, 288(22), 2836–2845. <https://doi.org/10.1001/jama.288.22.2836>
- Stokols, D., Hall, K. L., Taylor, B. K., & Moser, R. P. (2008). The science of team science. *American Journal of Preventive Medicine*, 35(2 Suppl), S77–S89. <https://doi.org/10.1016/j.amepre.2008.05.002>
- Provan, K. G., & Kenis, P. (2008). Modes of network governance. *Journal of Public Administration Research and Theory*, 18(2), 229–252. <https://doi.org/10.1093/jopart/mum015>
- Patel, V., Belkin, G. S., Chockalingam, A., Cooper, J., & Saxena, S. (2013). Integrating mental health services into primary care. *PLoS Medicine*, 10(6), e1001448. <https://doi.org/10.1371/journal.pmed.1001448>
- *Kroenke, K., & Unützer, J. (2017). Closing the false divide: Sustainable approaches to integrating mental health services into primary care. *Journal of General Internal Medicine*, 32(4), 404–410. <https://doi.org/10.1007/s11606-016-3967-9>
- *Schillok, H., Gensichen, J., Panagioti, M., Härter, M., Watzke, B., König, H. H., & von Wolff, A. (2025). Effective components of collaborative care for depression in primary care. *JAMA Psychiatry*, 82(9), 868–876. <https://doi.org/10.1001/jamapsychiatry.2025.0183>
- *Reiss-Brennan, B., Brunisholz, K. D., Dredge, C., Briot, P. C., Grazier, K., Wilcox, A., Savitz, L., & James, B. (2016). Association of integrated team-based care with health care quality, utilization, and cost. *JAMA*, 316(8), 826–834. <https://doi.org/10.1001/jama.2016.11232>
- McGinty, E. E., Baller, J., Azrin, S. T., Juliano-Bult, D., & Daumit, G. L. (2015). Interventions to improve communication between mental health and primary care. *Psychiatric Services*, 66(4), 423–434. <https://doi.org/10.1176/appi.ps.201400216>

Week Beginning November 15: Cultural & Contextual Adaptations

Culturally adapted ESTs, cultural responsiveness, dissemination barriers

- Bernal, G., & Domenech Rodríguez, M. M. (2012). Cultural adaptations of psychological treatments: A review and meta-analytic update. *Annual Review of Clinical Psychology*, 8, 213–239. <https://doi.org/10.1146/annurev-clinpsy-032511-143041>
- Barrera, M., & Castro, F. G. (2006). A heuristic framework for the cultural adaptation of interventions. *Journal of Community Psychology*, 34(2), 163–175. <https://doi.org/10.1002/jcop.20096>
- Huey, S. J., & Polo, A. J. (2008). Evidence-based psychosocial treatments for ethnic minority youth. *Journal of Clinical Child & Adolescent Psychology*, 37(1), 262–301. <https://doi.org/10.1080/15374410701820174>
- Chu, J., & Leino, A. (2017). Advancement of evidence-based treatments for diverse populations: A research agenda for cultural adaptation. *Behavior Therapy*, 48(3), 337–353. <https://doi.org/10.1016/j.beth.2016.09.005>
- Hall, G. C. N., Ibaraki, A. Y., Huang, E. R., Marti, C. N., & Stice, E. (2016). A meta-analysis of culturally adapted psychological interventions. *Journal of Consulting and Clinical Psychology*, 84(6), 511–525. <https://doi.org/10.1037/ccp0000145>
- *Lau, A. S. (2006). Making the case for selective and directed cultural adaptations of evidence-based treatments. *Clinical Psychology: Science and Practice*, 13(4), 295–310. <https://doi.org/10.1111/j.1468-2850.2006.00042.x>
- *Lau, A. S., Huey, S. J., & Baumann, A. A. (2023). Advances in the adaptation and implementation of evidence-based interventions for historically marginalized groups. *Behaviour Research and Therapy*, 168, 104377. <https://doi.org/10.1016/j.brat.2023.104377>
- *Cabassa, L. J., & Baumann, A. A. (2013). A two-way street: Bridging implementation science and cultural adaptations of mental health treatments. *Implementation Science*, 8, 90. <https://doi.org/10.1186/1748-5908-8-90>

Week Beginning November 22: Fall Break and Thanksgiving Holidays

Week Beginning November 29: Student Grant Proposal Presentations

Peer review panels, NIH-style critique

- *Pier, E. L., Brauer, M., Filut, A., Kaatz, A., Raclaw, J., Nathan, M. J., ... Carnes, M. (2018). Low agreement among reviewers evaluating the same National Institutes of Health grant applications. *Proceedings of the National Academy of Sciences*, 115(12), 2952–2957. <https://doi.org/10.1073/pnas.1714379115>
- *Li, D., & Agha, L. (2015). Big names or big ideas: Do peer-review panels favor stars? *Science*, 348(6233), 434–438. <https://doi.org/10.1126/science.aaa0185>
- *Gallo, S. A., Carpenter, A. S., & Glisson, S. R. (2013). Teleconference versus face-to-face peer review and the National Institutes of Health scoring system: A randomized trial. *PLOS ONE*, 8(8), e71693. <https://doi.org/10.1371/journal.pone.0071693>
- Fang, F. C., & Casadevall, A. (2016). Research funding: The case for a modified lottery. *mBio*, 7(2), e00422-16. <https://doi.org/10.1128/mBio.00422-16>
- Guthrie, S., Ghiga, I., & Wooding, S. (2017). What do we know about grant peer review in the health sciences? *F1000Research*, 6, 1335. <https://doi.org/10.12688/f1000research.11917.1>
- Ioannidis, J. P. A. (2014). How to make more published research true. *PLOS Medicine*, 11(10), e1001747. <https://doi.org/10.1371/journal.pmed.1001747>

Week Beginning December 6: Professional Communication

Clinical science audiences, case conferences, presentations

- Nosek, B. A., Ebersole, C. R., DeHaven, A. C., & Mellor, D. T. (2018). The preregistration revolution. *Proceedings of the National Academy of Sciences*, 115(11), 2600–2606. <https://doi.org/10.1073/pnas.1708274114>
- Cumming, G. (2014). The new statistics: Why and how. *Psychological Science*, 25(1), 7–29. <https://doi.org/10.1177/0956797613504966>
- Munafò, M. R., Nosek, B. A., Bishop, D. V. M., Button, K. S., Chambers, C. D., Percie du Sert, N., ... Ioannidis, J. P. A. (2017). A manifesto for reproducible science. *Nature Human Behaviour*, 1, 0021. <https://doi.org/10.1038/s41562-016-0021>
- *Boutron, I., Altman, D. G., Moher, D., Schulz, K. F., & Ravaud, P. (2017). CONSORT statement for randomized trials of nonpharmacologic treatments: A 2017 update and a CONSORT extension for nonpharmacologic trial abstracts. *Annals of Internal Medicine*, 167(1), 40–47. <https://doi.org/10.7326/M17-0046>
- *Alley, M. (2013). *The craft of scientific presentations: Critical steps to succeed and critical errors to avoid* (2nd ed.). New York, NY: Springer. <https://doi.org/10.1007/978-1-4419-8279-7>

Assignments and Exams

Grant Proposal: This capstone assignment is intended to prepare you to conceptualize, articulate, and defend a research project in the style of an NIH F31 proposal. The goal is to scaffold your ability to generate independent research ideas, integrate empirical literature, and articulate methodological rigor (skills that will be directly transferable to your dissertation and early career grant submissions). Draft a mini research grant proposal 7 to 10 pages (excluding references), single-spaced with 1-inch margins in Times New Roman Font, prepared in APA-style format that addresses a question *relevant to intervention science* (e.g., treatment mechanisms, moderators of outcome, comparative effectiveness, dissemination/implementation). Your proposal must integrate empirical evidence to justify design choices and anticipate potential limitations.

Oral Presentation of Grant Proposal: Students will deliver a 15-minute professional presentation of their proposal, followed by a brief 5-minute Q&A. The presentation serves at least two purposes: (1) it provides as an opportunity to obtain feedback and revise your proposal before submission for grading, (2) it reinforces skills needed for proposal defenses, professional conferences, and future academic job talks. Students are expected to prepare and deliver their presentation at a professional standard, including appropriate attire and the use of visual aids (e.g., slides, figures) that meet the expectations of academic and scientific audiences. Presentations must be completed within the 15-minute limit while adequately covering the specific aims and research strategy. Presentations that exceed 15 minutes will be penalized at a rate of 5 points per additional minute, up to a maximum of 15 points. Presentations that omit a required content area (in an effort to end within 15 minutes) will not only lose the points associated with that criterion but will also trigger an additional coverage

penalty of up to 15 points, reflecting the professional expectation that all essential domains must be addressed within the allotted time.

Presentations will be evaluated on a 30-point scale as follows:

Content Coverage (Clear aims = 3 pts; Clear strategy = 5 pts; Integration of mediators/moderators, cultural/contextual factors, and dissemination/implementation implications = 2 pts) = 10 points
Organization & Clarity (e.g., logical flow, effective signposting and transitions, audience can easily follow the arc of the proposal) = 5 points
Visual Aids (e.g., slides clear, not text heavy; visuals do not replace or distract from key points; design consistency) = 5 points
Delivery & Professionalism (e.g., clear, confident, appropriate pacing; audience engagement; professional demeanor) = 5 points
Q & A Responsiveness (e.g., able to defend choices & anticipates critiques, manages time well to allow comments) = 5 points

Penalties for exceeding time or failing to cover required domains will be deducted from the total score.

Written Grant Proposal Document: Proposals are due December 6th and should be submitted as a PDF in the following order:

1. Specific Aims (1 page)

2. Research Strategy (6–8 pages)

Significance: Rationale, contribution to literature, importance for clinical science.

Innovation: How the project challenges or refines existing approaches.

Approach: Detailed research design, methods, analytic strategy, and consideration of power/sample size.

Consideration of mediators/moderators, cultural/contextual factors, and potential dissemination/implementation implications.

3. References (no page limit).

At least 20 references, primarily primary-source empirical work.

Submitted proposals will be graded on a 70-point scale as follows:

*Clarity and feasibility of aims (15 points)
*Integration of literature and justification of approach (20 points)
*Innovation and significance (12 points)
*Methodological rigor (15 points)
*Writing quality and adherence to format (8 points)

Asynchronous, Self-paced Labs: Each lab requires demonstration of specific competencies (e.g., outcome monitoring) that are grounded in empirical research and best practices. Labs may be repeated as many times as necessary until the required competency threshold is met. Successful completion is determined by demonstrated mastery rather than time spent, reflecting the course's emphasis on evidence-based training, mastery learning, and skill acquisition consistent with a clinical science model. The following four labs must be accomplished (recommend budgeting 3-4 weeks per lab). Each lab is graded pass/fail. The lab

requirement counts for 20% of the final course grade. Students earn these points only if all four labs are passed. Passing fewer than four labs results in 0% awarded for the lab requirement.

Routine Outcome Tracking Lab

Selecting measures, administration, scoring, interpretation of common measures (e.g., BDI-II, BAI, PHQ-9, GAD-7, OQ 45.2, YBOC)

- Switzer, G. E., Wisniewski, S. R., Belle, S. H., Dew, M. A., & Schultz, R. (1999). Selecting, developing, and evaluating research instruments. *Social Psychiatry and Psychiatric Epidemiology*, 34(8), 399–409. <https://doi.org/10.1007/s001270050161>
- *Lutz, W., Schwartz, B., & Delgadillo, J. (2022). Measurement-based and data-informed psychological therapy. *Annual Review of Clinical Psychology*, 18, 71–98. <https://doi.org/10.1146/annurev-clinpsy-071720-014821>
- *Whipple, J. L., & Lambert, M. J. (2011). Outcome measures for practice. *Annual Review of Clinical Psychology*, 7, 87–111. <https://doi.org/10.1146/annurev-clinpsy-040510-143938>
- Muir, H. J., Coyne, A. E., Morrison, N. R., Boswell, J. F., & Constantino, M. J. (2019). Ethical implications of routine outcomes monitoring for patients, psychotherapists, and mental health care systems. *Psychotherapy*, 56(4), 459–469. <https://doi.org/10.1037/pst0000246>
- McAleavey, A. A., & Moltu, C. (2021). Understanding routine outcome monitoring and clinical feedback in context: Introduction to the special section. *Psychotherapy Research*, 31(2), 142–144. <https://doi.org/10.1080/10503307.2020.1866786>
- Reese, R. J., Duncan, B. L., & Clements-Hickman, A. L. (2024). Do practice-generated data improve psychotherapy effectiveness (routine outcome monitoring)? In F. T. L. Leong, J. L. Callahan, J. Zimmerman, M. J. Constantino, & C. F. Eubanks (Eds.), *APA handbook of psychotherapy: Evidence-based practice, practice-based evidence, and contextual participant-driven practice* (pp. 193–211). American Psychological Association. <https://doi-org.libproxy.utdallas.edu/10.1037/0000354-013>

Structured and Semi-structured Interviews Lab

Selecting, administering, scoring, and interpreting semi-structured/structured interviews (e.g., CAPS-5, M.I.N.I., SCID-5)

- *Weathers, F. W., Bovin, M. J., Lee, D. J., Sloan, D. M., Schnurr, P. P., Kaloupek, D. G., ... & Marx, B. P. (2018). The Clinician-Administered PTSD Scale for DSM-5 (CAPS-5): Development and initial psychometric evaluation in military veterans. *Psychological Assessment*, 30(3), 383–395. <https://doi.org/10.1037/pas0000486>
- *Sheehan, D. V., Lecrubier, Y., Sheehan, K. H., Amorim, P., Janavs, J., Weiller, E., Hergueta, T., Baker, R., & Dunbar, G. C. (1998). The Mini-International Neuropsychiatric Interview (M.I.N.I.): The development and validation of a structured diagnostic psychiatric interview for DSM-IV and ICD-10. *Journal of Clinical Psychiatry*, 59(Suppl 20), 22–33; quiz 34–57.
- Osório, F. L., Loureiro, S. R., Hallak, J. E. C., Machado-de-Sousa, J. P., Ushirohira, J. M., Baes, C. V. W., Apolinário da Silva, L., de Sousa, J. P. M., Fracon de Souza Crippa, J. A., & Crippa, J. A. S. (2019). Clinical validity and intrarater and test-retest reliability of the Structured Clinical Interview for DSM-5 – Clinician Version (SCID-5-CV). *Psychiatry and Clinical Neurosciences*, 73(12), 754–760. <https://doi.org/10.1111/pcn.12931>

- *Shankman, S. A., Funkhouser, C. J., Klein, D. N., Davila, J., Lerner, D., Hee, D., Rodriguez, S., & Krueger, R. F. (2018). Reliability and validity of severity dimensions of psychopathology assessed using the Structured Clinical Interview for DSM-5 (SCID). *International Journal of Methods in Psychiatric Research*, 27(1), e1590. <https://doi.org/10.1002/mpr.1590>

Objective Measures Lab

Selecting, administering, scoring, and interpreting objective measures (e.g., MMPI, PAI, CBCL)

- Sattler, J. M. (2022). Broad measures of behavioral, social and emotional functioning and of parenting and family variables. In *Foundations of behavioral, social, and clinical assessment of children*. (7th ed., pp. 327-372). Jerome M. Sattler, Publisher, Inc.
- Groth-Marnat, G., & Wright, A. J. (2016). Minnesota Multiphasic Personality Inventory. In G. Groth-Marnat & A. J. Wright (Eds.), *Handbook of psychological assessment* (6th ed., pp. 243-370). Wiley.
- Groth-Marnat, G., & Wright, A. J. (2016). Personality Assessment Inventory. In G. Groth-Marnat & A. J. Wright (Eds.), *Handbook of psychological assessment* (6th ed., pp. 371-416). Wiley.
- Groth-Marnat, G., & Wright, A. J. (2016). Millon Clinical Multiaxial Inventory. In G. Groth-Marnat & A. J. Wright (Eds.), *Handbook of psychological assessment* (6th ed., pp. 417-488). Wiley.
- Groth-Marnat, G., & Wright, A. J. (2016). NEO Personality Inventory. In G. Groth-Marnat & A. J. Wright (Eds.), *Handbook of psychological assessment* (6th ed., pp. 489-512). Wiley.

Therapeutic Process & Fidelity Lab

Rupture and repair, therapeutic alliance, interpersonal competence, treatment fidelity and integrity, therapist responsiveness, and boundaries of competence.

- Karpiak, C. P. & Norcross, J. C. (2022). Evidence-based therapy relationships. In G. Asmundson (Ed.) *Comprehensive Clinical Psychology* (2nd ed., Vol. 2, pp. 119-127). Elsevier. <https://dx.doi.org/10.1016/B978-0-12-818697-8.00100-X>
- Stirman, S. W., Gumpert, N. B., Calloway, A., et al. (2025). A comparison of pragmatic and scalable strategies to assess fidelity to cognitive processing therapy in routine care settings. *Behavior Therapy*, 56(3), 453–469. <https://doi.org/10.1016/j.beth.2024.12.003>
- Marques, L., Valentine, S. E., Kaysen, D., et al. (2019). Provider fidelity and modifications to cognitive processing therapy in a diverse community health clinic: Associations with clinical change. *Journal of Consulting and Clinical Psychology*, 87(4), 357–369. <https://doi.org/10.1037/ccp0000384>
- Peyser, D., Sysko, R., Webb, L., & Hildebrandt, T. (2021). Treatment fidelity in eating disorders and psychological research: Current status and future directions. *The International Journal of Eating Disorders*, 54(12), 2121–2131. <https://doi.org/10.1002/eat.23624>
- Cox, J. R., Martinez, R. G., & Southam-Gerow, M. A. (2019). Treatment integrity in psychotherapy research and implications for the delivery of quality mental health services. *Journal of Consulting and Clinical Psychology*, 87(3), 221–233. <https://doi.org/10.1037/ccp0000370>

- Herschell, A. D. (2010). Fidelity in the field: Developing infrastructure and fine-tuning measurement. *Clinical Psychology: Science and Practice*, 17(3), 253–257. <https://doi.org/10.1111/j.1468-2850.2010.01216.x>
- Zilcha-Mano, S., Eubanks, C. F., Bloch-Elkouby, S., & Muran, J. C. (2020). Can we agree we just had a rupture? Patient-therapist congruence on ruptures and its effects on outcome in brief relational therapy versus cognitive-behavioral therapy. *Journal of Counseling Psychology*, 67(3), 315–325. <https://doi.org/10.1037/cou0000400>
- Hatcher, R. L. (2015). Interpersonal competencies: Responsiveness, technique, and training in psychotherapy. *American Psychologist*, 70(8), 747–757. <https://doi.org/10.1037/a0039803>

Final Exam: The course concludes with a comprehensive take-home final examination designed to assess your ability to integrate and apply the major domains of intervention science covered during the semester. The exam is cumulative, independent, and closed to collaboration. Scholarly writing at a doctoral level is expected, with answers integrating empirical literature and methodological critique. Exams will be made available November 29 and are due December 13. The exam will consist of essay-style questions that require you to:

- Critically evaluate treatment-outcome methodologies, including RCTs, adaptive/dismantling designs, and meta-analytic approaches.
- Synthesize evidence on mechanisms, mediators, and moderators of therapeutic change.
- Compare and contrast psychotherapy models and empirically supported treatments (ESTs), including transdiagnostic, modular, and process-based approaches.
- Appraise dissemination and implementation frameworks, supervision research, and systems of care as they relate to the translation of intervention science into practice.
- Integrate cultural and contextual considerations into the design and evaluation of interventions.
- Demonstrate professional communication skills in framing arguments, evaluating evidence, and presenting methodological critiques.

Course Grades

Final grades will be determined by weighting the earned scores on the assignments and exams as follows:

Requirement	Weight in Final Grade
Class Participation	15%
Asynchronous Labs (Pass/Fail)	20%
Grant Proposal	30%
Final Exam	30%

Letter	Percentage	Letter	Percentage
A	93-100	C	73-76
A-	90-92	C-	70-72
B+	87-89	D+	67-69
B	83-86	D	63-66
B-	80-82	D-	60-62
C+	77-79	F	59 or lower

All assigned grades will adhere to the allocations above and will not be changed upon request. Any grade dispute must be raised within one week of the posting of the grade in question. Grades will be posted on eLearning.

Course Policies

Incompletes: Incompletes will only be given if the only remaining requirement is completion of the final exam. An incomplete grade would need to be arranged prior to the final exam and would require approval of the instructor based upon a substantial excuse.

Extra Credit: There is no extra credit in this course.

Late Work: A penalty of one letter grade will be assessed for each calendar day late.

Communication

This course utilizes online tools for interaction and communication. Some external communication tools such as regular email and a web conferencing tool may also be used during the semester. For more details, please visit the [Student eLearning Tutorials](#) webpage for video demonstrations on eLearning tools. Please use email or office hours to ask questions of the instructor. Student emails will be answered within 3 working days under normal circumstances.

Class Materials

The instructor may provide class materials that will be made available to all students registered for this class as they are intended to supplement the classroom experience. These materials may be downloaded during the course; however, these materials are for registered students' use only. Class materials may not be reproduced or shared with those not in class or uploaded to other online environments except to implement an approved AccessAbility Resource Center accommodation. Failure to comply with these University requirements is a violation of the [Student Code of Conduct](#).

Class Attendance

Regular and punctual class attendance is expected. Students who fail to attend class regularly are inviting scholastic difficulty.

Class Participation

Regular class participation is expected. Students who fail to participate in class regularly are inviting scholastic difficulty. A portion of the grade for this course is directly tied to your participation in this class. It also includes engaging in group or other activities during class that solicit your feedback on homework assignments, readings, or materials covered in the lectures (and/or labs). Class participation is documented by faculty. Successful participation is defined as consistently adhering to University requirements, as presented in this syllabus. Failure to comply with these University requirements is a violation of the [Student Code of Conduct](#).

Texas Senate Bill 17, the recent law that prohibits diversity, equity, and inclusion programs and activities at public universities in Texas, does not in any way apply to academic course instruction. Students should not feel the need to self-censor or limit their participation in

academic courses pertaining to topics of race and racism, structural inequality, LGBTQ+ issues, or diversity, equity, and inclusion, and related topics.

Class Recordings

Students are expected to follow appropriate University policies and maintain the security of passwords used to access recorded lectures. Unless the AccessAbility Resource Center has approved the student to record the instruction, students are expressly prohibited from recording any part of this course. Recordings may not be published, reproduced, or shared with those not in the class, or uploaded to other online environments except to implement an approved AccessAbility Resource Center accommodation. Failure to comply with these University requirements is a violation of the [Student Code of Conduct](#).

The instructor may record meetings of this course. These recordings will be made available to all students registered for this class if the intent is to supplement the classroom experience. If the instructor or a UTD school/department/office plans any other uses for the recordings, consent of the students identifiable in the recordings is required prior to such use unless an exception is allowed by law.

Academic Integrity / Misconduct

Students are reminded that academic misconduct will not be tolerated. For example: (a) copying another student's work, in part or in whole; (b) unauthorized communication to obtain information about a test(s); (c) using unauthorized material during a test; (d) handing in any work product that is not your own; (e) signing someone else's name; (f) failing to give credit to the intellectual property of another in any written work (including, but not limited to, plagiarizing). For a more extensive listing of behaviors that are considered misconduct please refer to the UT Dallas [Community Standards and Conduct](#).

Comet Creed

This creed was voted on by the UT Dallas student body in 2014. It is a standard that Comets choose to live by and encourage others to do the same:

"As a Comet, I pledge honesty, integrity, and service in all that I do."

Accommodations for Students with Disabilities

Please review [the section](#) within the UT Dallas Syllabus Policies and Procedures webpage.

Server Unavailability or Other Technical Difficulties

The University is committed to providing a reliable learning management system to all users. However, in the event of any unexpected server outage or any unusual technical difficulty which prevents students from completing a time sensitive assessment activity, the instructor will provide an appropriate accommodation based on the situation. Students should immediately report any problems to the instructor and contact the online [eLearning Help Desk](#). The instructor and the eLearning Help Desk will work with the student to resolve any issues at the earliest possible time.

Academic Support Resources

Please visit the [Academic Support Resources](#) page to view the University's academic support resources for all students.

UT Dallas Syllabus Policies and Procedures

The information contained in the following link constitutes the University's policies and procedures segment of the course syllabus. Please review the catalog sections regarding the [credit/no credit](#) or [pass/fail](#) grading option and withdrawal from class. Please go to [UT Dallas Syllabus Policies](#) webpage for these policies.

The descriptions and timelines contained in this syllabus are subject to change at the discretion of the Professor.

PCSAS Question 2: We need more (not comprehensive) information on how the initial two-year¹ practicum sequence will unfold, as well as the way the Assessment Clinic² will operate.

PCSAS Question 3: We need some (not comprehensive) information on when and how students will learn CBT (or whatever the primary evidence-based therapeutic modality will be in the clinic). If easier, you can incorporate this response into the prior response.

PSEC Operational Plan

Before detailing the initial two-year practicum sequence, we first provide context on the establishment and scaling of the PSEC to clarify when each phase of student training begins. This operational overview is followed by a semester-by-semester training sequence (outlined on pg. 3 thru 6), which includes specific activities for students in Years 1 through 3. Together, these sections respond to PCSAS Questions 2 and 3 by describing how training progresses, when and how students begin to see clients, and how the assessment clinic and primary treatment modality are introduced.

The PSEC is designed to scale over three years in alignment with cohort growth. With five students matriculating annually, full operations are not expected before the third cohort enters in fall 2028. Year 1 students will not carry cases but will engage in structured clinical learning and skill development through the PSEC. Formal practicum activity begins in Year 2, when students begin delivering both psychotherapy and assessment services under faculty supervision.

Supervision will initially be provided by the PSEC Director, with other clinically trained faculty available to support as needed. By Year 3 of clinic operations, we anticipate adding a second supervisor, likely with a focus on assessment, depending on the Director's interests. We have two current tenure-track and one instructional faculty with expressed interest in taking on supervisory roles, and we have approval for and will be hiring two tenure-track faculty in the coming years, with at least one if not both of those hires having the interest and qualifications to also take on a supervisory role. Therefore, staffing for supervision will be fully met within the first years of the PSEC operation.

At full operational strength (i.e., when three cohorts are enrolled), five Year 3 student clinicians will each carry two psychotherapy cases (10 clients total in the PSEC at any given time), and five Year 2 student clinicians will each complete approximately 3 integrated psychological evaluations annually (15 total across the PSEC). Year 1 students will not carry their own cases but will be actively engaged in observation, simulation, and co-therapy roles (on cases seen by the PSEC Director, which are not included in the total number of clients above). Referrals for

¹ Although we refer to Years 2 and 3 as the practicum years, students are active in the Psychological Services and Education Center (PSEC) throughout Years 1 to 3. We avoid calling Year 1 “pre-practicum,” as it implies a passive role. While students are only assigned cases in Years 2 and 3, all three years are part of the training sequence.

² Although we sometimes refer to the “Assessment Clinic” for internal workload tracking, all clinical training in both psychotherapy and assessment takes place within a single, unified training site: the Psychological Services and Education Center (PSEC). The term “practicum” is used only to distinguish supervision and course credit assignments and does not indicate separate operations or infrastructure.

both psychotherapy and assessment will come from trusted community partners and will be triaged to ensure students work only with diagnostically straightforward, low-complexity cases. Psychotherapy referrals will focus on common presenting concerns (e.g., mild to moderate anxiety, depression, adjustment), and psychological assessment cases will be selected for clarity and developmental fit with training goals. Exposure to more complex or comorbid presentations will occur through structured observation of sessions conducted by the PSEC Director and, subsequently, via external practicum placements.

PSEC Instructional Model and Rationale

Many clinical training programs continue to devote significant classroom time to introducing individual assessment measures and surveying psychotherapy models. In assessment, this often involves step-by-step instruction in procedures that are now widely accessible through publisher resources and video demonstrations. In psychotherapy, classroom time is frequently allocated to theoretical models with limited empirical grounding or whose observed effects are more likely explained by common factors (e.g., therapeutic alliance). These practices persist not because they are pedagogically superior, but because accreditation and licensure frameworks were developed in a pre-digital era when faculty-delivered, in-person instruction was the only feasible means of transmitting technical knowledge. “Training” became equated with completing a “course,” and that structure has continued to shape clinical education in ways that no longer reflect the reality of modern instructional design.

The UT Dallas concentration departs from this model by prioritizing deliberate practice. A growing body of evidence in both medicine and psychology demonstrates that structured, feedback-driven, and repetitive training yields superior outcomes in clinical skill development compared to traditional lecture-based instruction. Meta-analyses and randomized trials show that simulation-based training with deliberate practice improves both skill acquisition and long-term retention and has been formally endorsed by the American Heart Association.^{3,4,5} Similar effects have been observed in psychotherapy education, where deliberate practice improves observer-

³ Kanoksin, S., Songkhla, B. N., Tansawet, A., & others. (2025). Effectiveness of deliberate practices versus conventional lecture in trauma training for medical students. *BMC Medical Education*, 25(1), 130. <https://doi.org/10.1186/s12909-025-06732-8>

⁴ McGaghie, W. C., Issenberg, S. B., Cohen, E. R., Barsuk, J. H., & Wayne, D. B. (2011). Does simulation-based medical education with deliberate practice yield better results than traditional clinical education? A meta-analytic comparative review of the evidence. *Academic Medicine*, 86(6), 706–711. <https://doi.org/10.1097/ACM.0b013e318217e119>

⁵ Cheng, A., Nadkarni, V. M., Mancini, M. B., & others. (2018). Resuscitation education science: Educational strategies to improve outcomes from cardiac arrest: A scientific statement from the American Heart Association. *Circulation*, 138(6), e82–e122. <https://doi.org/10.1161/CIR.0000000000000583>

rated performance more than theoretical teaching alone.^{6,7,8} We apply this principle to both assessment and psychotherapy training to maximize applied learning and developmental impact.

Our goal is to ensure students get the best of our faculty, not the least common denominator of our collective expertise. Rather than sharing baseline level foundational knowledge that can be acquired outside of the classroom, course time is reserved for conceptual reasoning, evidence integration, and applied clinical thinking. In short, foundational knowledge is treated the same way we approach research literacy: students are expected to build core knowledge, skills, and abilities outside the confines of coursework so they can participate fully in supervision, contribute meaningfully to teams, and develop toward autonomous clinical and scholarly practice. This is not experimental. For ~75 years (i.e., post-Boulder), clinical psychology has relied on students independently learning analytic tools, reviewing methods, and observing expert researchers at work, often outside the structure of formal coursework or credit-bearing instruction. Clinical training should follow the same principle.

To support this model, we will host weekly non-credit-bearing “watch parties” facilitated by the PSEC Director. Students are expected to participate throughout years 1 thru 3. These sessions function like lab meetings or journal clubs and will feature recorded psychotherapy conducted by the Director or curated professional tutorials on key assessment tools.⁹ Students are not evaluated by attendance or quiz performance but by what they can demonstrate in supervision, clinical documentation, fidelity checks, assessment checkouts, and real-time decision-making. While our approach is a departure from traditional classroom-based instruction, it aligns with our commitment to developmental, competency-based education. Importantly, it also mirrors the realities of professional life (where one must take ownership and accountability for professional development) and equips students to stay current, curious, and capable throughout their careers.

PSEC Training Sequence (Years 1–3)¹⁰

What follows is a representative semester-by-semester outline of how training in psychotherapy and assessment may typically unfold across the three years of training in the PSEC. Please note that instruction in Process-Based CBT (PB-CBT) and measurement-based care (MBC) begins in Year 1, Semester 1 and continues throughout the student’s time in the PSEC. This clinical training is further supported by formal instruction in CPSY 7310 (Clinical Psychological Science II), which reinforces foundational concepts and their application in practice. Although PB-CBT

⁶ Østergård, O. K., Nielsen, J., Hjorthøj, C., Nilsson, K. K., & Poulsen, S. B. (2022). Effect of deliberate practice training of graduate students in general psychotherapeutic skills compared with theoretical teaching: Protocol for the randomised controlled TRIPS trial. *BMJ Open*, 12(9), e062506. <https://doi.org/10.1136/bmjopen-2022-062506>

⁷ Westra, H. A., Norouzian, N., Poulin, L., & others. (2021). Testing a deliberate practice workshop for developing appropriate responsivity to resistance markers. *Psychotherapy*, 58(2), 175–185. <https://doi.org/10.1037/pst0000311>

⁸ Diamond, G., Wlodek, B., Arthey, S., & Parker, S. (2025). A systematic review of deliberate practice in psychotherapy: Definitions, operationalization, and preliminary outcomes. *Psychotherapy*, 62(2), 113–131. <https://doi.org/10.1037/pst0000569>

⁹ Pearson, for example, provides a complete WAIS-5 training video series: <https://www.youtube.com/playlist?list=PLTRNT7i5iG3Sh0e3t-pMhiIithwBggQIH>

¹⁰ Non-comprehensive; illustrative activities.

is not explicitly mentioned in Semesters 4-6, students are expected to utilize PB-CBT with their patients and will therefore engage in ongoing training, supervision, and implementation.

Year One - Fall (Semester 1)

Example Psychotherapy Training Activities

- Attend watch parties introducing Process-Based CBT (PB-CBT), functional analysis, and measurement-based care (MBC)
- Observe PB-CBT sessions conducted by the PSEC Director
- Complete weekly simulations focused on common (e.g., ruptured alliance) and high-intensity clinical challenges (e.g., suicide risk, mandatory reporting)
- Participate in discrete in-session tasks (e.g., mood checks, homework review)

Example Assessment Training Activities

- Attend watch parties introducing WAIS, WISC, WIAT, WRAT, and other measures
- Practice administration using dedicated testing kits

Year One – Spring (Semester 2)

Example Psychotherapy Training Activities:

- Continue attending watch parties introducing Process Based CBT (PB-CBT), functional analysis, and measurement-based care (MBC)
- Co-lead one PB-CBT case with the PSEC Director
- Use symptom and process data to inform session planning and case formulation
- Continue participating in simulations with feedback
- Draft progress notes for co-therapy sessions
- Draft treatment plan for co-therapy course of treatment

Example Assessment Training Activities:

- Continue attending watch parties introducing WAIS, WISC, WIAT, WRAT, and other measures
- Practice structured interviewing techniques (e.g., M.I.N.I., CAPS-5)
- Practice Mental Status Exams (MSE)
- Read sample reports
- Practice writing background sections
- Draft intake report for co-therapy session
- Draft discharge report for co-therapy session

Year One – Summer Interlude

- Complete administration and scoring competency checkouts for assessment measures.
- Complete a closed-book CBT knowledge examination to ensure mastery of foundational principles before advancing to independent psychotherapy cases in Year 2. This exam is based on two required (free) texts (*A Provider's Guide to Brief Cognitive Behavioral Therapy*, Cully et al., 2020; and *Cognitive Behavioral Therapy Strategies*, Crane & Watters, 2021) and evaluates student knowledge of the CBT model, agenda setting, homework, core cognitive and behavioral strategies, relapse

prevention, and cultural adaptations. The exam includes multiple-choice, short-answer, and case vignette items.

- **CBT Knowledge Examination Policy:** If a student does not pass the examination on the first attempt, the failure is documented and the student must complete a structured remediation plan before re-examination. Remediation plans are individualized but typically include directed readings, written study questions, and faculty consultation to review conceptual gaps. Students may retake the exam only after completing this remediation plan. Repeated failure to pass the exam will trigger a formal faculty review of the student's progress and may result in additional remediation requirements or discontinuation from the practicum sequence.

Year Two – (Semester 3)

Example Psychotherapy Training Activities:

- Begin the semester as the primary therapist for one low-complexity case
- Increase caseload to two low-complexity clients by mid-semester, based on supervisor approval
- Independently develop case formulations and treatment plans using PB-CBT
- Implement outcome tracking and process-based measurement
- Participate in weekly individual and group supervision

Example Assessment Training Activities:

- Practice writing Results and Summary/Conclusions section based on provided data
- Study frameworks for conceptualization (e.g., DSM-5, HiTOP, process-based models)

Year Two – (Semester 4)

Example Psychotherapy Training Activities:

- Continue working with two cases; complete at least one full treatment arc
- Deliver collaborative outcome reviews
- Submit final treatment summaries and visualizations of client progress

Example Assessment Training Activities:

- Conduct intake, interview, and MSE for first real assessment case
- Select and administer a full testing battery
- Prepare second integrated report under faculty supervision
- Provide client feedback as part of the assessment process

Year Two – Summer Interlude

- Complete four self-paced online training courses in empirically supported treatments: CPTWeb2.0 (Cognitive Processing Therapy; <https://cpt.musc.edu>), TF-CBTWeb2.0 (Trauma-Focused Cognitive Behavioral Therapy; <https://tfcbt2.musc.edu>), CBT-I Web (Cognitive Behavioral Therapy for Insomnia; <https://cbtiweb.org>), and Acceptance-Based Behavioral Therapy for GAD (<https://lms.mghcme.org/GAD>). Each program consists of multiple interactive modules covering the foundations and

delivery of the intervention, includes video demonstrations and practice resources, and provides a certificate of completion. Students must register, complete all four trainings, and submit proof of registration plus certificates of completion no later than September 1st. Students are responsible for their own registration and payment for these trainings. The collective cost is offset by the concentration's extensive use of freely provided primary source readings and correspondingly limited use of textbooks in the clinical psychological science core courses.

Year Three – (Semester 5)

Example Psychotherapy Training Activities:

- Continue with two cases with increased autonomy
- Adjust treatment mid-course based on data and client response
- Prepare externship application materials (e.g., case summaries, supervisor letters)

Example Assessment Training Activities:

- Begin and complete second assessment
- Prepare second integrated report under faculty supervision
- Provide client feedback as part of the assessment process

Year Three – (Semester 6)

Example Psychotherapy Training Activities:

- Continue treatment with increased focus on supervision use and ethical reflection
- Finalize externship materials with faculty input
- Transition caseload (drop to one or zero cases to transition to externship)

Example Assessment Training Activities:

- Initiate third and final required assessment
- Select and apply additional measures as appropriate to referral questions
- Incorporate interpretive synthesis into results and conclusions
- Demonstrate full assessment cycle competence
- Prepare summary materials for external review or internship application

PCSAS Question 4: *We need more information about how the concentration-wide competency-based approach will be presented to students to help us see that this will be workable for students at a concentration-wide level.*

Following attainment of preliminary accreditation, we will launch a public-facing website that will introduce prospective applicants to our COMETS (Competency-Oriented Methods of Education, Training, and Supervision) framework in broad, student-friendly terms. As shown in Appendix A, this page highlights our training philosophy and the integration of coursework, research, and clinical training, helping prospective students begin to see how our approach may differ from more traditionally structured programs. Because the site is not yet publicly visible, Appendix A includes a full-page image to convey the gestalt of the webpage's structure, followed by readable screenshots of each section to provide a closer look. This webpage is meant to spark interest and reflection. We intentionally keep it conceptual and high-level, reserving deeper structural detail for those who join the program.

Appendix B includes our Competency Companion Guide for Clinical Psychological Science, which is a student-facing document that we will introduce at orientation (and potentially during interview day, if appropriate). This guide translates the COMETS framework into a clear, domain-by-domain roadmap, outlining expectations, evaluation methods, and developmental milestones across the six-year training sequence. Our hope is to directly address the workability concern raised in the review. Our hope is that by presenting COMETS in this format, students can see not just the *what* of their training, but also the *why* and *how*. The guide is intended to act as a navigational tool they can revisit as they progress through the program.

PCSAS Question 5: *We need to know how all students will be able to access more advanced statistical training than they will receive in the two-course series they complete in their first year.*

Students have access to a range of advanced quantitative and methodological electives, including coursework in longitudinal modeling, multivariate analysis, statistical machine learning, neural net modeling, and computational methods, among others. Our curriculum is intentionally streamlined to allow students the flexibility to select electives that align with their research interests and developmental needs. We consider this flexibility a strength of our training approach. Students work closely with their mentors and advisors to select appropriate electives.¹

A list of advanced statistical and methods electives available to our students appears below.

PSYC 7313 (ACN 7313 and HCS 7313) The General Linear Model (3 semester credit hours) This course reviews basic concepts in statistical analysis and focuses on simple and multiple regression

¹ As instructed, our self-study focused on the structure of the concentration and did not include a comprehensive list of available electives. However, all Psychology PhD students, including those in the Clinical Psychological Science concentration, are required to complete two courses in another doctoral psychology concentration (i.e., Developmental Psychology, Cognitive Psychology, or Social and Personality Psychology), and may pursue additional electives as needed to support their individual training goals.

and Analysis of Variance (ANOVA)-related statistical techniques in a behavioral science context. Special emphasis is placed on implementing and interpreting complex ANOVA designs (e.g., nested ANOVA, factorial ANOVA involving multiple between-subjects and within-subjects factors).

HCS 5314 (ACN 5314) Computational Modeling Methods in Behavioral and Brain Sciences (3 semester credit hours) Historical introduction to machine learning algorithms from a cognitive-neuroscience perspective. Includes an introduction to important and widely used computational modeling methodologies in psychology, neuroscience, and machine learning. No mathematical prerequisites and no computer programming prerequisites, but students will use machine learning software to support data analyses and simulation experiments.

HCS 6310 (ACN 6310) Fundamentals of Functional Brain Imaging (3 semester credit hours) In-depth topics in brain imaging including neuroimaging detection systems (primarily MRI), experimental design, statistical techniques in image analysis, clinical applications of functional neuroimaging, and reviews of pertinent literature using functional brain imaging to illuminate various cognitive and perceptual processes, including language, memory, hearing, and vision.

HCS 6110 (ACN 6110) Fundamentals of Functional Brain Imaging Lab (1 semester credit hour) This course covers the neurophysiology and mechanics of functional neuroimaging data collection and analysis. Topics include methods of data collection, experimental design, data analysis, and interpretation. Students work in the lab to develop proficiency with neuroimaging analysis software tools. Class meetings will consist of lectures, hands-on demonstrations, and work-through sessions with readily available data sets to learn the mechanics of basic fMRI data analysis.

HCS 6348 (ACN 6348) Neural Net Mathematics (3 semester credit hours) Vector calculus, Radon-Nikodym density functions, vector calculus-based probability theory, and Markov random fields with machine learning and artificial neural network modeling applications. Emphasizes applications of theory to unsupervised, supervised, and reinforcement learning machines and deep learning with MATLAB software implementations. This course is a required prerequisite for **ACN 6349** and **HCS 6349**.

HCS 6349 (ACN 6349) Statistical Machine Learning (3 semester credit hours) Mathematical tools for investigating the asymptotic behavior of both batch and adaptive machine learning algorithms including convergence of gradient descent batch learning algorithms convergence of adaptive stochastic approximation learning algorithms, and convergence of Monte Carlo Markov Chain algorithms. M-estimation and bootstrap asymptotic statistical theory for characterizing asymptotic behavior of parameter estimates as a function of sample size to support model selection, specification analysis, and hypothesis testing. Emphasizes applications of theory to unsupervised, supervised, and reinforcement learning machines and deep learning with MATLAB software implementations.

HCS 7310² (ACN 7310) Advanced Research Methods (3 semester credit hours) Advanced methods of inquiry and analysis unique to cognition and neuroscience, communication sciences and disorders, or psychological sciences. May be repeated for credit as topics vary (12 semester credit hours maximum).

PSYC 7313 (ACN 7313 and HCS 7313) The General Linear Model (3 semester credit hours) This course reviews basic concepts in statistical analysis and focuses on simple and multiple regression

² *HCS 7310 is a topics-based course that can be repeatable. One of our faculty members teaches Dyadic Data Analysis under this course name/number, and other faculty members can use this course to offer novel advanced statistical methods courses.*

and Analysis of Variance (ANOVA)-related statistical techniques in a behavioral science context. Special emphasis is placed on implementing and interpreting complex ANOVA designs (e.g., nested ANOVA, factorial ANOVA involving multiple between-subjects and within-subjects factors).

HCS 7317 Longitudinal Research Methods (3 semester credit hours) Advanced methods course covering issues related to the design and analysis of multiple waves of data over time. Topics include measurement and attrition, panel models, latent growth curve modeling, and time-varying and invariant predictors. Designed for students interested in development, variability in processes over time, or experimental change.

HCS 7320 (ACN 7320) Topics in Multivariate Data Analysis using R (3 semester credit hours) R programming language (including writing functions and using special packages). Using the R programming language to analyze standard designs used in Behavioral and Brain Science. Includes designing publication ready graphics and analysis of experimental data and surveys. May be repeated for credit as topics vary (9 semester credit hours maximum).

HCS 7321 (ACN 7321) Topics in Multivariate Data Analysis Theory (3 semester credit hours) Principal component analysis, correspondence analysis, multidimensional scaling, discriminant analysis, partial least square methods, multi-table analysis, cluster analysis, and various other statistical techniques. Includes discussion of computationally intensive cross-validation inference methods such as jackknife and bootstrap. May be repeated for credit as topics vary (9 semester credit hours maximum).

PCSAS Question 6: *Finally, we need clarification regarding whether the concentration plans to train only students with adult clinical and research interests and how this will be communicated to students, if so. Alternatively, we need to know how all students with developmental psychopathology interests will receive adequate training.*

We appreciate the opportunity to clarify this point. The Clinical Psychological Science concentration at UT Dallas will not limit training to students with adult-only clinical or research interests. We intend to admit students with adult, child, or lifespan developmental interests, depending on research fit and mentorship alignment.

We recognize that Child Psychopathology is a commonly expected course in clinical science training. While it was not highlighted in our original materials, it is already offered in our department:

PSYC 6335 (HCS 7376 and HD CD 6385) Child Psychopathology (3 semester credit hours) Childhood psychopathology manifested during infancy through adolescence. Normal personality development as a basis for identifying psychopathology. Issues of etiology, diagnosis, prognosis, and social policy.

In addition, the Department of Psychology offers an MS in Human Development and Early Childhood Disorders as well as a Developmental Psychology concentration within the Psychology PhD program. As a result, students in the Clinical Psychological Science concentration have access to a broad catalog of electives in developmental science, assessment, and intervention. Furthermore, all Psychology PhD students (see Appendix C of the original self-study submission, “Psychology PhD Student Guide”) are required to take two courses outside

their primary concentration from another doctoral area (i.e., Developmental Psychology, Cognitive Psychology, or Social and Personality Psychology). Students with developmental psychopathology interests will likely fulfill this requirement by selecting courses from the Developmental Psychology concentration, in addition to relevant electives. Collectively, these courses support training in developmental theory, assessment tools, family systems, culture, neurobiology, trauma, and other key areas relevant to developmental psychopathology.

Importantly, we also have approval to continue a tenure-track faculty search focused on child psychopathology. We conducted a successful search this past academic year and identified a highly qualified finalist. Unfortunately, after a lengthy negotiation period, the candidate ultimately declined the offer due to a spouse's reluctance to relocate. Because the decision came late in the recruitment cycle, we were unable to proceed with an alternate candidate. However, we are resuming the search in the upcoming academic year and remain committed to hiring in this area to further strengthen our training capacity in developmental psychopathology.

We did not include a full list of electives in our self-study, as our understanding was that the focus should remain on required coursework and overall concentration structure. We recognize now that this may have given the impression that developmental coursework is limited. In reality, our offerings are extensive and ready to support students with developmental interests.

Moving forward, we will ensure that the availability of developmentally focused training is communicated clearly through advising, public-facing materials, and orientation resources. We are committed to supporting both adult- and child-focused clinical scientists, and our curriculum is intentionally designed to be flexible enough to accommodate these varied interests.

A list of relevant developmental courses appears below.

PSYC 6335 (HCS 7376 and HD CD 6385) Child Psychopathology (3 semester credit hours) Childhood psychopathology manifested during infancy through adolescence. Normal personality development as a basis for identifying psychopathology. Issues of etiology, diagnosis, prognosis, and social policy.

PSYC 6368 (ACN 6368 and HCS 6368) Language Development (3 semester credit hours) Advanced study of normal oral language development. The goals of this course are to consider the developmental trajectories of the different components of language; to consider the varied and critical roles of language in human development; to understand the impact of culture, different languages, child factors and the environment on development; and to be introduced to the theoretical perspectives driving research and thinking in this area of inquiry.

HD CD 6310 Working with Parents and Caregivers (3 semester credit hours) Skills needed by professionals to work with parents and caregivers of children with special needs. Topics address skills needed to assess parent strengths and resources, and to assist parents in understanding and promoting their children's development and adjustment; including effective communication techniques, and strategies to enhance parental efficacy to advocate for their children.

HD CD 6312 Atypical Development (3 semester credit hours) Disorders of development from conception through preschool. Effects of developmental impairments on the child, family, and the environment and prognosis and treatment options.

HDCD 6315 Assessment Theory (3 semester credit hours) Latest developments in the field of assessment with young children, including behavioral observation, contextual multi-faceted assessment, and inclusion of the family. Training in traditional psychometrics and assessment tools/techniques.

HDCD 6316 Developmental Assessment (3 semester credit hours) Training in administration of play-based, curriculum-based, and specialized development domain assessment tools. Training in use of Zero-to-Three diagnostic classification system (DC: 0-5) for differential diagnosis decision-making. Emphasis on clinical judgment/observation, interpretation, and integration with assessment results.

PSYC 6357 (**HCS 6357** and **HDCD 6319**) The Developing Child: Infants and Toddlers (3 semester credit hours) Empirical data and theories related to development in multiple content domains (e.g., cognitive, social, motor, language, physical) from the prenatal period through 24 months of age. Emphasis is placed on effects of how changes in behaviors in one domain affect functioning in other domains and viewing the child within a larger ecological system.

PSYC 6320 (**HCS 6359** and **HDCD 6320**) The Developing Child: Toddler and Preschool Years (Two to Five Years) (3 semester credit hours) Developmental milestones of 24-to 60-month olds across several domains, the mechanisms of developmental change, individual differences in development, social influences on development, and the practical applications of research on early child development.

HDCD 6330 Families and Culture (3 semester credit hours) The purpose of this seminar is to investigate interactions between family, culture and child development. Topics will include intercultural competence, cultural identity and biases, intercultural communication, and cultural variations in family values and practices. The impact of the students' own culture, attitudes, and beliefs in working with families from diverse backgrounds will be emphasized.

HDCD 6335 Intervention Paradigms (3 semester credit hours) Historical, theoretical, practice, and research bases for early intervention paradigms. Research methods to evaluate the effectiveness of early intervention programs and determine evidence-based practice.

HDCD 6345 Trauma-Informed Care (3 semester credit hours) Explores various types of traumas, including acute, chronic, systemic, historical, vicarious, and complex developmental trauma, as well as its effects on the development of children, youth, and families. Emphasis will be placed on trauma-informed practices for professionals, with a special focus on building a trauma-informed lens for future work.

HDCD 6351 Play Matters (3 semester credit hours) Study of the importance of play for child development and its unique value as an evaluative and remedial tool in early intervention. Combines didactic and experiential elements to facilitate an integrated understanding of play-based approaches to working children and families. Includes guided experience facilitating a playgroup program for young children and their parents.

HDCD 6352 Optimal Learning Environments (3 semester credit hours) Explores design of developmentally appropriate physical environments to optimize learning and development for infants through young children. Topics include initial setup, ability to modify, and ways to overcome barriers associated with creating ideal spaces.

HDCD 6355 Family Outreach and Assessment (3 semester credit hours) This practicum is designed to provide students, as part of a 2-3 person team, supervised experiences with young children from

diverse backgrounds, at selected off-campus sites. Emphasis is on assessment, professional report writing, communicating assessment results orally to the families and other professionals, the referral process, and working together in small teams.

HDCD 6360 Behavior Management (3 semester credit hours) Observational methodology in behavioral assessment and a review of principles and procedures of behavior change from social learning and applied behavior analysis perspectives. Particular attention will be given to the design, implementation, and evaluation of behavioral interventions with children and families.

HDCD 6365 (COMD 7336) Social Communication in Early Childhood Disorders (3 semester credit hours) Examines the development of infant and toddler social attention and communication in the first two years of life, and assessment/intervention strategies for identifying and treating children with social attention and communication challenges (i.e., autistic children). The course is divided into two main sections. The first section focuses on introductory topics, including autism, what constitutes evidence-based practices, and historical perspectives on intervention. The remainder of the course is devoted to understanding a) social development in young children and relevant intervention for each stage, and b) the relationship among social development, social communication, and language.

HDCD 6370 Intervention with Young Children (3 semester credit hours) Emphasis on methods and procedures for facilitating development of high risk, developmentally delayed young children through relationship-based intervention. Reviews the contributions and perspectives of various early intervention models and disciplines. Students design and implement individualized intervention treatment plans.

HDCD 6390 Infant Mental Health (3 semester credit hours) Infant Mental Health is a global field of study that examines the healthy development of children ages 0 - 3 years. This course discusses theoretical foundations within the context of relationships that support early child development. Self-reflection is a primary practice through the duration of this course and class activities are designed to provide this opportunity. Content discussion includes infant brain development, temperament, early childhood trauma, attachment and separation, and parent attachment styles. The course text identifies intervention programs and early childhood disorders which are also highlighted in this course.

HDCD 6395 Medical and Biobehavioral Factors in Early Childhood Disorders (3 semester credit hours) The normal functioning of organ systems and the most common malformations, dysfunction, and diseases. Effects of these disorders on the child and family.

HCS 7387 (ACN 7387 and COMD 7387) Developmental Neurobiology of Language and Cognition (3 semester credit hours) Consideration of current neurological data concerning the pre/postnatal development of the brain and how changes in brain structure and function provide the foundations of children's language development and language disorders. We will examine models of the neural substrates and circuitry underpinning developmental changes in language, cognitive control and working memory, episodic memory, and visual face processing in both typical (monolingual and bilingual) language users and in children with developmental language disorders including specific language impairment, developmental language disorders, reading disabilities, autism spectrum disorder, and cognitive-communicative language disorders.

COMETS Framework

Start: [Clinical Psychological Science](#)

In This Section

COMETS Framework

Scientific Mentorship

Admissions and Funding

Competency-Oriented Methods of Education, Training, and Supervision (COMETS)

The COMETS Framework defines the structure of doctoral training in Clinical Psychological Science at UT Dallas. It organizes coursework, research, and clinical training using evidence-based methods that foster lasting expertise. While many traditional models emerged under historical accreditation structures and philosophical divides, COMETS was designed to reflect current scientific priorities and how expertise develops in practice. It represents a purpose-built framework for training the next generation of clinical scientists. Each part of the framework reflects a deliberate design choice: it is modern, engineered for expertise, wholly integrative, and optimized for impact.

COMETS at a Glance



Modern by Design

COMETS was created without the constraints of legacy standards or entrenched training assumptions. Its structure reflects current priorities in clinical science. From coursework to clinical progression, every element is aligned with evidence-based principles and the realities of doctoral-level training.



Engineered for Expertise

COMETS leverages learning science and discipline-specific research on how clinical science competencies develop during doctoral training. Coursework and training are sequenced for developmental fit and strengthened through focused skill-building, timely feedback, and structured progression.



Wholly Integrative

COMETS aligns admissions, mentorship, coursework, research, evaluation, and clinical development into a cohesive system. This integration strengthens connections across training domains and prepares students to think rigorously, adapt flexibly, and advance clinical science with lasting impact.



Optimized for Impact

COMETS is designed to help today's students become tomorrow's leaders in clinical psychological science. Its structure supports early research productivity, personalized mentorship, and evidence-driven clinical training to prepare future colleagues who will shape the field through scholarship and impact.

Coursework



Structured for Depth, Calibrated for Breadth

All students complete a two-course foundational sequence in clinical psychology, followed by an assessment that identifies whether additional coursework is needed in areas such as psychopathology, assessment, or psychotherapy. Students who demonstrate readiness in a given area may pivot earlier into specialized training aligned with their developing scholarly expertise.



Flexible, Field-Connected Electives

In addition to electives in clinical psychological science, students may deepen expertise through coursework in [Developmental, Cognitive, or Social/Personality Psychology](#), or expand into [BBS areas](#) such as Cognitive Neuroscience, Behavioral Neuroscience, and Communication Sciences and Disorders.



Coursework as Scientific Apprenticeship

Courses build intellectual agility through tasks modeled on the work of scientific professionals. Students strengthen theory-method integration, refine analytic reasoning, and develop scientific writing through a mix of structured and open-ended assignments, with opportunities for direct feedback and iterative improvement.



Guided by a Committee, Not a Checklist

Each student works with a personalized training committee that helps shape a coherent plan of study. The committee ensures course selection and pacing stay aligned with scholarly development and provides structured oversight that supports expertise across both core competencies and emerging interests.

Research Training



Mentorship Anchored from Day One

Research training begins with mentor-matching at admission and is strengthened by the guidance of a three-member Personalized Training Committee. This committee offers continuity across the student's training arc and complements the mentor's research guidance with additional faculty perspectives on coursework, professional development, and scholarly trajectory.



Research That Builds a Pipeline

Students begin research from the outset. Training emphasizes the skills needed to manage multiple research streams, guiding projects from idea through IRB, data collection, analysis, presentation, and publication. Producing approximately eight peer-reviewed publications by year two serves as a benchmark for developing the pace and pipeline required for research-intensive careers.



Scholars in the Making

Workshops, seminars, and structured mentorship target essential skills in grant writing, scientific writing, data analysis, and professional communication. Students practice these skills repeatedly, across multiple contexts and projects, to build fluency, confidence, and the habits of inquiry that support lasting scholarly contributions.



Embedded in Research Communities

Research training is embedded within the intellectual ecosystems of UT Dallas and the School of Behavioral and Brain Sciences (BBS). Students benefit from the McDermott Library, Office of Research, and [BBS seven interdisciplinary research centers](#)—each providing methodological expertise, collaborative opportunities, and access to specialized populations.

Clinical Training



Competency First, Always

Clinical training begins in the Psychological Services and Education Center (PSEC), where students receive closely supervised training in assessment and intervention. [Advances in quantitative neurology](#) inform case conceptualization to guide interventions grounded in [Process-Based CBT](#). Students progress from simulated tasks to working with patients only after demonstrating the skills required to provide effective care. This competency-contingent progression reflects our ethical commitment to patients and facilitates professional self-efficacy.



Measurement at the Core

Students are trained to improve care, not just deliver it. [Measurement-based care \(MBC\)](#) is fully integrated into all psychological services, with students using standardized tools (e.g., QD-45.2, BDI-II, WAI-SF) to guide treatment, track progress, and inform supervision. Clinical reasoning and supervision are expected to be data-informed, with MBC outcomes embedded in both decision-making and documentation, which is treated as a professional writing skill.



Externships with Intent

The Dallas-Fort Worth metroplex supports a wide range of training contexts. The program prioritizes relationships that are mutually beneficial, placing students in settings that serve medically underserved populations in federally designated shortage areas. [Learn more about HPSEs](#). Accrual of clinical hours is carefully monitored to ensure access to high-intensity supervision and genuine research engagement.



Skills that Serve Science

Clinical training is essential to developing an evidence-driven understanding of assessment and intervention that directly informs research design, data interpretation, and translational relevance. While many students also complete [APPC externships](#), internship is not required for graduation. Students pursuing academic or research-intensive careers may, with committee approval, transition to another concentration to complete their PhD in Psychology. Students interested in licensure are advised to consult state-specific requirements: [ASPPB Licensure Information](#).



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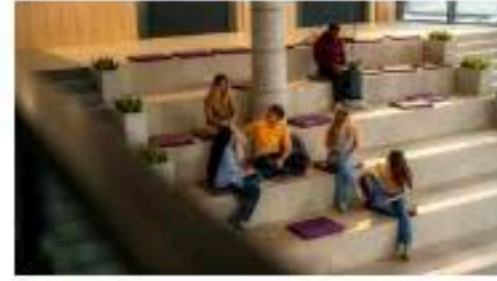
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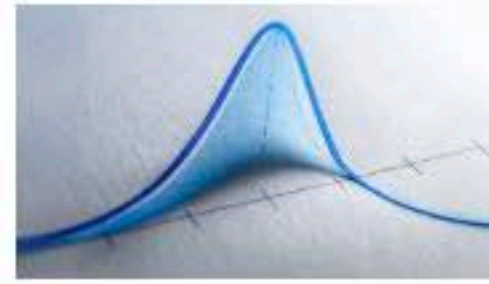
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Competency Companion Guide for Clinical Psychological Science

This guide outlines the nine core competency domains that frame your doctoral training in Clinical Psychological Science at UT Dallas. Framed by Competency-Oriented Methods of Education, Training, and Supervision (COMETS), your growth across the six-year doctoral training journey is structured, intentional, and integrated across coursework, research training, and clinical experiences. Each competency domain is deliberately woven into your educational experience to ensure you develop the knowledge, skills, and professional identity needed for successful practice as a clinical scientist. Through the COMETS model, you will receive structured feedback, develop across sequenced training stages, and gain increasing independence as you prepare for a career that bridges science and service. Here, service is defined broadly as the application of clinical science to improve public health through research, system leadership, and evidence-based innovation.

Understanding the COMETS Framework

COMETS are deliberately designed to be visible, integrated, and actionable across all domains of your doctoral training. Rather than existing as a separate administrative tool, the COMETS framework is woven into your entire educational experience through clinical work, coursework, research, and advising.

1

Visible Framework

The competency model appears explicitly in learning outcomes, clinical supervision forms, research mentorship, and evaluation rubrics. You'll encounter the same language and expectations across all training contexts.

2

Integrated Approach

Competencies shape the structure of your curriculum, clinical training, and evaluation. The framework is referenced consistently in advising, supervision, and progress reviews to create a cohesive training experience.

3

Actionable Feedback

You'll receive structured feedback on competency development in multiple settings using rubrics aligned with the nine core domains. This feedback helps you track progress and identify areas for growth.

4

Developmental Sequence

Training is intentionally sequenced to reflect developmental progression across competencies, from foundational knowledge to increasingly autonomous practice in clinical, research, and professional domains.

Through consistent exposure, aligned feedback, and developmentally sequenced training, you'll experience this competency-first approach not as a bureaucratic overlay, but as a living framework that guides your growth as a clinical scientist. This integration ensures you understand expectations, receive coherent support, and are well prepared for increasingly autonomous roles across training settings.

Research Competency Development

Research is the first of our nine competency domains because it is foundational to all others. We believe it is difficult to engage meaningfully in ethical practice, assessment, intervention, or supervision without a deep understanding of how scientific knowledge is created. The ability to design studies, evaluate methods, and interpret data enables you to move beyond surface-level application and develop a more flexible, critical, and independent stance toward science itself. By learning to build knowledge from the ground up, you become equipped to understand the science of *anything*.

That is a powerful place to stand. We are not simply preparing you to be smart in six years. We are launching you into a multi-decade career moving at the speed of science. The COMETS framework exists to ensure that you are ready to lead from the front, not only reading and implementing what others create, but identifying what is missing, envisioning what comes next, and building it.



Throughout your training, research competency development is integrated with clinical work through evidence-based practice, outcome measurement, and the application of scientific reasoning in applied settings. Faculty will evaluate your progress using behavioral anchors that assess your ability to critically evaluate evidence, design methodologically sound studies, analyze data appropriately, and communicate findings effectively.

Ethical and Legal Standards & Individual and Cultural Diversity

Ethical and Legal Standards

This competency domain ensures you develop a thorough understanding of ethical principles, legal standards, and professional guidelines that govern psychological practice. You'll learn to recognize ethical dilemmas, apply ethical decision-making models, and maintain appropriate professional boundaries in both clinical and research contexts.

- *Review* empirical and professional literature on ethical decision-making and legal frameworks.
- *Evaluate* ethical challenges in assessment and intervention research (e.g., control conditions, informed consent, risk in trials).
- *Apply* structured ethical decision-making models to research design, case conceptualization, and dissemination.
- *Complete* ethics training (CITI, HIPAA) and understand relevant ethical codes, (governing research, human subjects protections, and patient care).
- *Contribute* to scholarship on ethics in clinical science.
- **Where developed:**
 - **CPSY 7300** → labs on informed consent, risk assessment, confidentiality; Integrative Psychopathology Analysis paper requires attention to ethical implications of nosology.
 - **CPSY 7310** → Week 1 on Measurement-Based Care (framed as an ethical imperative); Grant Proposal requires discussion of ethical safeguards in intervention research.
 - **PSEC practicum** → ethical dilemmas in real-time supervision.
 - **Portfolio/dissertation** → IRB training and protocols.

Individual and Cultural Diversity

This domain focuses on developing awareness, knowledge, and skills in working with individuals and communities from varied backgrounds. You'll learn to recognize how your own background influences your professional work and develop competence in adapting your approach based on context, in both clinical work and scientific practice.

- *Examine* research on health disparities, cultural bias in measurement, and cultural adaptations of interventions.
- *Evaluate* equity implications of diagnostic and nosological frameworks.
- *Apply* diversity science in designing studies, interpreting findings, and adapting evidence-based practices.
- *Contribute* to scholarship by engaging in diversity-focused research and dissemination.
- **Where developed:**
 - **CPSY 7300** → Multicultural Considerations week; paper/presentation requires integrating cultural applicability of nosological frameworks.
 - **CPSY 7310** → Multicultural frameworks; Grant Proposal requires explicit integration of cultural/contextual moderators.
 - **Labs** → Core Clinical Skills V (cross-cultural microskills; broaching/bridging).
 - **Externships** → exposure to diverse populations; supervision feedback.

Both of these competency domains are assessed through direct observation, supervision evaluations, presentations, and written work. Faculty will look for evidence of your ability to apply ethical principles and attend to diversity considerations in both clinical and research settings. These competencies are cultivated through the COMETS framework, which sequences training, integrates applied feedback, and supports your development across increasingly complex contexts.

Professional Values, Attitudes, and Behaviors

This competency domain focuses on developing your professional identity as a clinical scientist and cultivating the values, attitudes, and behaviors essential for effective practice across both clinical and research settings. It encompasses professionalism, self-reflection, openness to feedback, and personal and proactive development as a clinical scientist.

Competencies:

- *Review* scholarship on professional identity formation, self-care, and ethical professionalism.
- *Apply* reflective practice to academic, clinical, and research development.
- *Evaluate* professional behaviors in clinical science contexts (publication ethics, authorship, peer review, patient care, consultation).
- *Contribute* to the professionalization of the field through dissemination and service.

Where developed:

- **CPSY 7300** → emphasis on philosophy of science, professionalism in early labs.
- **CPSY 7310** → Module on Professional Communication; cumulative exams assess professional-level synthesis.
- **Portfolio/dissertation** → reflective statements, scholarly writing.
- **Colloquia/conferences** → presenting scholarship with professionalism.

This competency is assessed through direct observation of your behavior in professional settings, supervision evaluations, peer and faculty feedback, and your ability to manage professional responsibilities. Faculty will look for evidence of your commitment to lifelong learning, ability to engage in self-care, and development of a coherent professional identity. These values are nurtured through the COMETS framework, which supports your growth through aligned expectations, structured feedback, and increasingly autonomous roles.

Communication and Interpersonal Skills

The Communication and Interpersonal Skills competency domain focuses on developing your ability to communicate effectively with the public, policy-makers, patients, colleagues, supervisors, and other professionals. This includes written and verbal communication, active listening, rapport building, and the ability to navigate difficult conversations.



Competencies:

- *Review* and *apply* empirical research on communication processes, alliance, and interdisciplinary collaboration.
- *Evaluate* effectiveness of scholarly communication across modalities (oral, written, visual).
- *Produce* professional-level presentations and manuscripts that integrate clinical science scholarship.

Where developed:

- **PSYC 7308** → Integrative Psychopathology Analysis *oral presentation* (20-min professional talk, graded on clarity, prioritization, pacing).
- **CPSY 7310** → Grant Proposal requires NIH-style writing; labs include fidelity monitoring and case presentation practice.
- **Final exams (PSYC 7308, CPSY 7300, CPSY 7310)** → essay-based, requiring scholarly written communication.
- **Department colloquia/PSEC** → case conferences and research presentations.

This competency is assessed through direct observation of your verbal and written communications (including both scholarly writing and clinical notes/reports) and via feedback from supervisors, instructors, mentors, and peers. Faculty will look for evidence of your ability to communicate clearly and effectively across settings, including clinical interactions, interdisciplinary collaborations, and scientific contexts such as lab meetings, manuscript preparation, and conference presentations. You'll be expected to adapt your communication style to the audience and situation, maintaining professionalism and clarity throughout. The COMETS framework supports your growth in this area through sequenced expectations, applied feedback, and opportunities for improvement.

Assessment Competency Development

Across your research training, you will critically engage with measurement theory, psychometric principles, and evaluation strategies to inform study design, data analysis, and interpretation. Whether selecting validated scales, evaluating standardized metrics, or analyzing patterns of individual differences, you will apply assessment science to generate knowledge that is both generalizable and clinically meaningful. The COMETS framework ensures this alignment is deliberate, reinforcing your development across investigative, translational, and applied roles.

The Assessment competency domain focuses on developing your ability to critically evaluate, design, and apply methods for measuring psychopathology. Training emphasizes psychometric rigor, construct validity, and cultural fairness, alongside integration of multiple data sources to advance case conceptualization and research design. From the outset, you will engage in simulation and structured evaluation of assessment tools, progress to advanced assignments such as the **Psychometrics Project in CPSY 7300**, and culminate in the ability to conduct and disseminate validation research, integrate measurement science into intervention studies, and contribute to innovations in psychological assessment.

Competencies:

- *Review* and synthesize empirical literature on measurement of psychopathology (validity, reliability, invariance, cultural generalizability).
- *Critically evaluate* instruments and structured methods with respect to psychometric properties and scientific grounding.
- *Apply* advanced psychometric methods to analyze measures.
- *Design* and propose validation studies that address gaps in psychopathology assessment science.
- *Contribute* to scholarship by disseminating empirical findings on measurement.

Where developed:

- **PSYC 7308** → Integrative Psychopathology Analysis paper requires integration of epidemiology, biological/cognitive mechanisms, environment, and nosological frameworks.
- **CPSY 7300** → Psychometrics Project (apply factor analysis, reliability, validity); outcome measurement weeks emphasize construct validity and measure evaluation; Final Exam tests ability to critique assessment science.
- **Labs** → Core Clinical Skills (e.g., fidelity in assessment, PVTs/SVTs).
- **Dissertation research** → often includes measure selection, validation, or adaptation.

Assessment competency is evaluated through structured assignments (e.g., the **Psychometrics Project in CPSY 7300**, the **Integrative Psychopathology Analysis in PSYC 7308**, and course-based final exams), research products (e.g., portfolio papers, dissertation proposals, and empirical manuscripts), and applied training in the PSEC. Faculty will assess your ability to critically evaluate the empirical evidence for assessment instruments, apply psychometric theory to data analysis, and design and disseminate validation studies. Evaluation also includes your ability to administer and score assessments with fidelity in the PSEC, synthesize findings into integrated case conceptualizations, and use assessment data to inform evidence-based treatment planning. Competence is further demonstrated by your ability to communicate assessment science effectively through written products (papers, proposals, reports) and oral dissemination (professional talks, case conferences, defenses).

- ❗ Prior to externship, you should be able to independently administer, score, and interpret common psychological tests, write integrated reports, and provide feedback to patients under supervision. During internship, you should demonstrate mastery of the full assessment process, including complex cases and specialized measures relevant to your area of focus.

Intervention Competency Development

Intervention competency is evaluated through structured assignments (e.g., the **Grant Proposal in CPSY 7310**, cumulative **Final Exams** in both core courses, and oral presentations), research products (e.g., empirical manuscripts, portfolio papers, and dissertation studies), and faculty-supervised activities. Faculty will assess your ability to critically appraise intervention literatures, evaluate methodological rigor in outcome and process research, and design proposals that test mechanisms, moderators, and mediators of change. Evaluation also includes your ability to apply advanced quantitative methods to intervention data, integrate findings from dissemination and implementation science, and communicate intervention scholarship effectively through professional writing (e.g., NIH-style proposals, manuscripts) and oral dissemination (conference-style presentations, defenses, case conferences).

What Counts as an Intervention?

Applying evidence to produce meaningful change, whether in people, systems, behavior, or ideas.

Interventions might include psychotherapy, experimental manipulations, tool development, or implementation strategies. What matters is that they are intentional, evidence-informed, and aimed at impact.

Competencies:

- *Review* and critique outcome literatures and comparative effectiveness research.
- *Evaluate* methodological rigor of intervention trials, including fidelity, control conditions, and outcome metrics.
- *Design* intervention studies testing mechanisms, mediators, moderators, and dissemination strategies.
- *Apply* quantitative methods (e.g., mediation/moderation models, longitudinal data) to evaluate mechanisms of change.
- *Contribute* to scholarship on intervention science through study design, proposal, and dissemination.

Where developed:

- **CPSY 7310** → core arc on outcome methods, moderators/mediators, ESTs, PB-CBT, and dissemination/implementation science; Grant Proposal assignment requires proposing an empirical study; Final Exam integrates across intervention science.
- **Summer interludes** → Year 1 CBT exam; Year 2 online EST certifications.
- **Labs** → fidelity monitoring, rupture repairs, outcome tracking.
- **PSEC practicum** → students implement ESTs while being evaluated on data-informed care.
- **Dissertation research** → often tests moderators/mediators, processes, or adaptations.

Intervention competency is assessed through structured scholarly products (e.g., the **Grant Proposal in CPSY 7310**, cumulative **Final Exams**, and oral presentations), research outputs (e.g., empirical manuscripts, dissertation projects, and conference presentations), and applied training in the PSEC. Faculty will evaluate your ability to critically review intervention literatures, appraise methodological rigor in treatment outcome research, and design intervention studies that test mechanisms, moderators, and mediators of change. Evaluation also includes your ability to deliver evidence-based treatments with fidelity in the PSEC, adapt interventions based on session-by-session outcome data, and integrate empirical findings into treatment planning and case conceptualization. Competence is further demonstrated by your ability to communicate intervention scholarship effectively in both written formats (proposals, manuscripts, case reports) and oral formats (professional presentations, case conferences, defenses).

Supervision Competency Development

The Supervision competency domain focuses on developing your understanding of supervision models and practices, both as a recipient of supervision and, eventually, as a provider of supervision. This competency ensures you can effectively utilize supervision for your own professional growth and develop the skills needed to supervise others in the future.

Competencies:

- *Review* empirical literature on supervision models and outcomes.
- *Evaluate* supervision processes in terms of effectiveness and alignment with evidence-based practice.
- *Apply* supervision models in clinical and research training contexts.
- *Contribute* to scholarship on supervision and training practices.

Where developed:

- **CPSY 7310** → late semester unit on supervision, drawing on empirical supervision research.
- **PSEC supervision** → students first as supervisees, later engaging in peer consultation.
- **Externships/internship** → progressive development of supervisory competencies.

Supervision competency is assessed through your engagement in supervision sessions, implementation of supervisory feedback, reflective assignments on supervision models, and (when applicable) evaluations of your emerging supervisory skills. Faculty will look for evidence of your ability to use supervision effectively for professional development, understand different supervision models, and develop the knowledge and skills needed for future supervisory roles.

This competency extends to both clinical and research settings. You may be asked to provide informal supervision or mentorship within research labs, including training junior students on data protocols, analytic tools, or authorship practices. The COMETS framework supports your development in this area by sequencing opportunities to first receive, then observe, and ultimately provide supervision aligned with your evolving professional responsibilities.

Consultation and Interdisciplinary Skills

The Consultation and Interdisciplinary Skills competency domain focuses on developing your ability to critically evaluate, apply, and disseminate psychological science in interdisciplinary contexts. Competence in this domain requires not only the ability to collaborate with other professionals to address complex problems, but also the ability to translate empirical findings, evaluate the evidence base for consultation models, and contribute to research on team-based care and systems-level interventions. Training in this domain emphasizes both scholarship and application: students will examine research on consultation and interdisciplinary collaboration in **CPSY 7310**, practice consultation skills through **PSEC supervision and case presentations**, and engage in interdisciplinary team science through **externships, colloquia, and dissertation research**.

Competencies:

- *Review* research on models of consultation and interdisciplinary collaboration.
- *Evaluate* evidence for effectiveness of consultation approaches in health and academic systems.
- *Apply* consultation models in research and clinical science contexts.
- *Contribute* to interdisciplinary scholarship and dissemination.

Where developed:

- **CPSY 7310** → professional communication module includes interdisciplinary reporting; final exam may include consultation scenarios.
- **PSEC** → case presentations with supervisors simulate consultation.
- **Externship** → interprofessional team experiences.
- **Colloquia** → communication of psychological science to broader academic audiences.

Throughout your training, you'll have opportunities to develop consultation skills in various contexts, including:

- **Clinical Settings:** Consulting with medical professionals, teachers, or other mental health providers about client care
- **Research Contexts:** Providing methodological or statistical consultation to research teams
- **Interdisciplinary Teams:** Collaborating with professionals from diverse disciplines to address complex patient needs

Consultation and interdisciplinary skills are evaluated through structured assignments (e.g., **final exams and professional communication tasks in CPSY 7310**), research products (e.g., proposals and manuscripts that include consultation or interdisciplinary components), and applied training contexts (e.g., **case conferences in the PSEC, externship team participation, and departmental colloquia**). Faculty will assess your ability to review and critique empirical literature on consultation and interdisciplinary collaboration, analyze models of team-based care, and apply findings in real-world contexts. Evaluation also includes your ability to communicate psychological science clearly to non-specialist audiences, contribute to interdisciplinary problem-solving, and disseminate consultation outcomes through professional presentations and written products. Competence is demonstrated when you can integrate psychological expertise into broader systems while maintaining scientific rigor and cultural sensitivity.

Competency Evaluation and Progress Monitoring

Throughout your training, your development across all nine competency domains will be regularly evaluated using a variety of methods. These evaluations are designed to provide you with clear feedback about your progress and to identify areas for continued growth.

Formal Evaluation Methods

- **Supervision Evaluations:** Feedback from clinical and research supervisors using structured rubrics
- **Annual Reviews:** Program-wide review of progress across domains by your advisor
- **Milestone Assessments:** Evaluation of performance at key transition points (e.g., qualifying exams, dissertation proposal, dissertation defense)
- **Course Assignments, Projects, and Exams:** Competency-aligned assessments in coursework, including writing, presentations, and applied analysis
- **Direct Observation:** Live or recorded observation of clinical work, lab presentations, or team contributions
- **Research Mentorship Feedback:** Evaluation of your contributions to research design, data analysis, writing, and scientific collaboration
- **Product Review:** Evaluation of written work (e.g., manuscripts, assessment reports, case conceptualizations, or feedback summaries)

Evaluation Criteria

You will be evaluated not just on your acquisition of knowledge, but on your ability to apply knowledge in contextually appropriate, ethical, and scientifically grounded ways. Evaluations are guided by behavioral anchors and developmental rubrics that reflect COMETS-aligned expectations around both clinical and research roles. Faculty use shared rubrics and behavioral anchors to ensure consistency across evaluators and settings.

Criteria are typically framed using a scale:

- **Below Expectations:** Performance significantly below what is expected at your training level
- **Approaching Expectations:** Skills are emerging but not yet consistently demonstrated
- **Meets Expectations:** Consistent performance appropriate for your current training level
- **Exceeds Expectations:** Performance that surpasses expectations for your level of training

Evaluations are intended to help you track progress, identify goals, and engage in continuous professional development. Feedback from these evaluations will be shared with you regularly through supervision sessions, written evaluations, and annual reviews. The COMETS framework ensures that evaluations are sequenced developmentally, integrated across settings, and aligned with your evolving roles as a scholar, clinician, and scientist. You will be assessed not just as a student of psychology, but as an emerging contributor to the field.

- 📌 Remember that competency development is a continuous process. Even after graduation, you'll continue to refine and expand your competencies throughout your professional career.

Internship Integration & Competency Synthesis

As you embark on your internship year, the culmination of your doctoral training, you are expected to demonstrate independent and integrated functioning across all nine competency domains. This signifies not only the ability to apply your skills with minimal supervision but also to embody the identity of a clinical scientist in complex, real-world settings, bridging research and practice seamlessly.

The COMETS framework supports this transition by ensuring that your competencies have developed in sequence and across contexts, preparing you to act ethically, flexibly, and scientifically in applied settings. Key expectations for your internship year include a holistic demonstration of expertise in the following areas:



Assessment

Independently conduct diagnostic evaluations, integrate complex psychometric and clinical findings, and communicate nuanced recommendations clearly to patients and interdisciplinary teams.



Intervention

Deliver evidence-based interventions to address the needs of patients or systems with autonomy, adjusting approaches based on routine outcome monitoring, process data, and patient or contextual feedback.



Ethics & Diversity

Uphold full ethical accountability and legal standards while demonstrating mature cultural competence, humility, and sensitivity in all professional interactions, ensuring equitable care for all.



Actively engage in scholarly dissemination through activities such as job talks, manuscript submission, or leading projects that evaluate mechanisms, improve systems, or generate translational impact in clinical or non-clinical domains.



Supervision & Consultation

Contribute psychological expertise to interdisciplinary case conferences and offer effective psychological consultation across various systems and settings.



Professional Identity

Function as a reflective, self-aware, and responsible clinical scientist, capable of making independent, evidence-informed decisions that integrate scientific knowledge with clinical practice.

Your performance during internship will be rigorously evaluated based on your ability to integrate these domains in a cohesive, ethical, and scientifically grounded manner. This comprehensive assessment supports not only optimal patient care but also your continued professional development as an emerging leader in clinical psychological science.

Whether your work involves individual patients, community systems, policy design, or scientific discovery, internship is your opportunity to synthesize your training and demonstrate how you enact the identity of a clinical scientist.

Integrating Competencies Across Training Contexts

A key strength of the competency-based model is how it creates connections across different aspects of your training. Rather than experiencing coursework, research, and clinical training as separate silos, the competency framework helps you see how skills developed in one context transfer to and enhance your performance in others. This integrative model reflects the COMETS philosophy, where competencies are deliberately scaffolded across settings to support increasingly sophisticated, cross-domain application.

Coursework Integration

Your academic courses are designed to build foundational knowledge and skills across competency domains. Learning outcomes and assignments are explicitly linked to relevant competencies, helping you see how classroom learning prepares you for applied work. For example, research methods courses develop skills relevant to the Research competency, while also building Assessment competencies through measurement selection and evaluation.

Clinical Training Integration

Across PSEC and externship placements, you'll apply competencies in coordinated ways. Assessment draws on technical skill, communication, cultural awareness, ethical judgment, and data-informed decision-making. Intervention builds on clinical reasoning, outcome monitoring, responsiveness to context, and an understanding of how change processes work. These experiences strengthen your ability to think scientifically in real time and apply your skills flexibly in complex settings.

Research Integration

Beyond developing Research skills, you'll also apply Ethical standards in IRB protocols, use Communication skills in manuscript and presentation work, and consider representativeness implications in your research design and dissemination. Many students also engage in research that informs assessment, advances intervention science, or evaluates system-level outcomes, reinforcing the bidirectional integration of research and practice.

As you progress through the program, you'll increasingly be expected to demonstrate integration across competency domains. By your internship year, you should be able to seamlessly blend scientific thinking, ethical awareness, cultural competence, and professional values into all aspects of your work.

The competency model serves as a unifying framework that helps you develop not just as a collection of skills, but as a whole professional with a coherent identity as a clinical scientist. By linking domains across research, practice, and professional development, COMETS prepares you to lead change across a wide range of settings.

We encourage you to actively reflect on these connections throughout your training. Consider keeping a journal of how skills developed in one setting transfer to others, and discuss these observations with your advisor and supervisors. This integrative thinking will strengthen your development across all competency domains and prepare you for the complexities of professional practice.